The My4MLife Logbook

Month 1

> Begin with the end in mind.
> Don’t lose your identity and your dignity while you still have a choice.

Dr. TJ Mundheim *Founder · My4MLife*

© 2026 My4MLife. All rights reserved.

How to use this logbook

This is not a book to read. It is a **logbook** to run.

You will write in it. You will mark up the trackers. You will tear out the scorecards and tape them to your bathroom mirror — or screenshot them for your phone lock-screen. The whole point is to convert the protocol from something you read about into something you actually run, day after day, with a pen in your hand and a face in your head.

The logbook covers **Month 1** — the first four weeks of the My4MLife cohort protocol. Each week builds on the prior one. You do not skip ahead. You do not start with Week 3 because hormones sound interesting. The order is the order on purpose.

The shape of the month

- **Part 1 — Your Why.** Before any protocol, you name the face you're doing this for and write your why. This page comes first because the protocol that you actually run beats the protocol that was theoretically better — and the protocol that you actually run is the one you run for a specific named person.
- **Part 2 — The 4M Framework.** A two-page anchor on Mind, Muscle, Mitigate, Motivate. Refer back to this any time the work starts to feel like a pile of tasks instead of a system.
- **Parts 3–6 — The Four Weeks.** Each week has the same shape: an opening frame, daily check-ins (one page per day), a weekly Zoom prep page, an end-of-week 4M reflection, and an adherence scorecard. Week 1 builds the foundation. Week 2 adds the lift. Week 3 audits hormones and the canaries — the ED canary for men, the perimenopause/menopause canary for women. Week 4 strips the insulting behavior, synthesizes the month, and writes the 30-day plan forward.

- **Part 7 — The Stack Reference.** One page per product. The full My4MLife stack — every OTC, every Rx tier, every dose, every pairing — in one place. Return here any time you forget why you're taking something.
- **Part 8 — The Advanced Layer (Optional).** The peptide tier, brain training, biofeedback, sauna / cold / grounding, the annual lab panel. Read it now if you want. **Do not deploy any of it until the foundation has been locked for 90 days.** The person who layers this on a broken foundation gets the bill, not the benefit.
- **Part 9 — Resources & Next Steps.** The 20-question MindSpan assessment as reference, the Rx consult routing table, the weekly Zoom rhythm, the 90-day plan template, and a closing letter.

How this logbook fits the app

This logbook gives you the month's work, week by week. The app adds one thing on top: a single **Move** the whole cohort tracks together, for twelve weeks. Neither replaces the other. In Month 1:

Week	This logbook	The app's Move
1	Foundation	Start your gut protocol, with your first meal, every day
2	Add the Lift	Hold the 9–6 eating window 5 of 7 days
3	Hormones and the Canaries	Protein-first, 30–40g at every fast-break
4	Mitigate, Synthesize, Recommit	Pantry purge: zero seed oils in the house

Everything you run here stays in place while the Move changes. The retake in Part 9 is your day-30 read; the app asks for a formal retake in Week 12.

Before you start

Read Part 1 with a pen in your hand. Do not skim. Do not "fill it in later." The exercises in Part 1 are short on purpose so that the only excuse for not doing them is that you are choosing not to. Write the face. Write the why. Write the baseline. Then turn the page.

The next time you open this logbook, you should already be someone who has named what they're protecting.

Who This Logbook Is For

Alzheimer's is a disease that shows up when you're older. It starts in midlife.

That one sentence is why this program starts where it starts. The damage that gets a name in your seventies is laid down in the decade you are most likely standing in right now — the years when blood pressure creeps, sleep stops holding, the waist moves, hearing dulls, the drinking gets normalized, and the hormones begin to drift, and nobody calls any of it a disease yet. So this logbook is not written for the person already worried about a diagnosis. It is written for the person in the decade it starts, who still has every lever available.

I am a man, and I have spent thirty years in practice treating patients of both sexes. That is the honest origin of this program: one clinician's voice, shaped by three decades of sitting across from people who waited too long.

The medicine is for everyone. The gut you repair, the sleep you defend, the muscle you keep, the mind you protect — none of it checks your driver's license first. Every page here is yours to run.

Where the physiology actually differs by sex, this logbook says so out loud instead of pretending it doesn't. Week 3 is the only place that splits: men run the ED-canary track, women run the perimenopause/menopause track, and everyone runs the decision tree at the end of it. Everywhere else — the gut, the sleep, the lifts, the food, the environment, the drinking, the purpose, the mind itself — the biology is shared, and so is the page.

— Dr. TJ

Begin with the end in mind.

Part 1 — Your Why

> Begin with the end in mind.
>
> Don’t lose your identity and your dignity while you still have a choice.

Before the protocol — a note from Dr. TJ

I’m going to ask you to do something counter-intuitive before I hand you a single supplement, a single lift, a single feeding window, a single piece of the 4M system.

I’m going to ask you to put the logbook down for ten minutes and look at a face.

Not your own face in the mirror. Somebody else’s face. Your spouse or partner. One of your kids — by name. Your aging father in the chair he doesn’t get out of anymore. The grandchild who isn’t born yet. Your own younger self at twenty-two. One specific human being whose life is materially worse if your mind goes before your body does.

I’ve been doing this work long enough to tell you what separates the people who finish a twelve-week cohort from the ones who quit in week three. It isn’t IQ. It isn’t income. It isn’t how clean the protocol is or how expensive the stack is. It’s whether they could name, on day one, who they were doing this for.

Here is the lock-in phrase for this entire logbook. Write it down. Read it back:

> The protocol that you run beats the protocol that was theoretically better. And the protocol that you run is the one you run for a specific named person.

The people who run it have a face. The ones who don't, drift.

That's why Part 1 is not about gut repair, or hormones, or sleep architecture, or resistance training, or any of the things you came here to fix. Part 1 is about you naming — in your own handwriting, on this page, today — the human being whose presence in your life makes the next twelve weeks non-negotiable.

If you skip this section and jump to the protocol, the protocol will not hold. I have watched this happen for years. Someone buys the stack, reads the chapters, runs week one beautifully, hits a bad Tuesday in week two, and quits. They quit because there was no face on the other side of the quitting. The quitting cost them nothing.

We are going to make the quitting cost something. We are going to give it a face and a name and a six-month outcome you wrote down with your own hand.

Do the four exercises in order. Don't optimize. Don't pretty it up. Use a pen, not a keyboard. Mistakes are fine — cross them out and keep going. This is the only section of the logbook that has no right answer and no wrong answer. There is only your answer.

Exercise 1 — Name the accountability target

This is the locked My4MLife rule and it is the foundation of everything that follows: **you must name a specific face before you write your why.** Not "my family." Not "the people I love." Not "myself." A specific human being I could pick out of a photograph if you showed me one.

Most people want to skip this step. Most people want to write “my family” because it feels generous and complete. It isn’t. “My family” is an abstraction and abstractions don’t get you out of bed at 5:45 in the morning when your knees hurt and the coffee isn’t on yet. A face does.

Pick one. You can add more later. Right now, name the single person whose deterioration in your absence — or whose loss of you to cognitive decline — is the thing you cannot accept. Write the name. Write what they call you. Write one sentence about what they need from you that nobody else on earth can give.

1. The face I’m doing this for is (name + relationship):

2. What this person calls me:

3. What this person needs from me that nobody else can give:

4. The thing I am most afraid this person will witness if I do nothing:

Exercise 2 — Write the Why

Now that you have the face, write the why. In your own handwriting. 150 to 300 words. No keyboard, no dictation, no AI assist for this one — your hand on the page is part of the imprint.

Use the prompts below as scaffolding. You don't have to answer every one in order. You don't have to use them at all if your why comes out cleanly without them. But if you stall, walk down the list.

When you are done, sign it and date it. Re-read it once a week for the next twelve weeks. Photograph it. Tape a copy to your bathroom mirror. This is the contract between today's version of you and the version of you who is going to want to quit in week six.

Prompts to draw from:

- If I do nothing for the next ten years, the most likely outcome is…
- The version of me my [named person] deserves looks like…
- What I want them to be able to say about me at my funeral is…
- The legacy I am building — not the money, the imprint — is…
- The reason I am unwilling to lose my mind before my body is…
- I am doing this so that on the day I die, my last clear thought is…

My Why (150-300 words, in my own handwriting):

Signed: _____ Date: _____

Exercise 3 — Six-month outcome targets

A why without a target is a wish. We are going to give your why three measurable outcomes, dated six months out from today.

The rules: each outcome must be (a) specific, (b) measurable with a number or a 1-10 score, and (c) something you can verify on the date without anybody else's permission. Not "feel better." Not "be healthier." A number. A score. A waist size. A weight. A lift. A morning erection frequency. Nights broken by sweats or a 3 AM wake-up. A sleep score. A cognitive clarity rating you write down today and re-rate in 180 days.

Pick from the menu below or write your own. Three is the cap. People who write ten targets hit zero. People who write three hit two.

Sample target categories (pick any three or write your own):

- Body weight (lb): _____ today → _____ in 6 months
- Waist circumference at navel (in): _____ today → _____ in 6 months
- Morning erection frequency (per week, honest): _____ today → _____ in 6 months
- Hot flashes or night sweats (days per week, honest): _____ today → _____ in 6 months
- Libido (1-10, honest): _____ today → _____ in 6 months
- Sleep score (1-10, or wearable score): _____ today → _____ in 6 months
- Cognitive clarity (1-10, honest): _____ today → _____ in 6 months
- Resting heart rate (bpm): _____ today → _____ in 6 months
- Heaviest compound lift (squat / hinge / press, lb): _____ today → _____ in 6 months
- Alcoholic drinks per week: _____ today → _____ in 6 months
- Mood / drive (1-10, honest): _____ today → _____ in 6 months

My three locked targets — six months from today:

Today's date: _____ Target date: _____

Target 1 (metric + today's number + 6-month number):

Target 2 (metric + today's number + 6-month number):

Target 3 (metric + today's number + 6-month number):

Exercise 4 — Starting baseline self-assessment

The last piece of Part 1 is the honest snapshot. We need a baseline. Without a baseline you cannot tell, twelve weeks from now, whether the work worked.

This mirrors the 20-category structure of the MindSpan Assessment, question for question, so your day-one baseline lines up with the retake in Part 9. Rate each category 0 to 5 — where 0 means *no issue at all in this area* and 5 means *daily problem affecting my life*. Then in one line, name the most current, most specific symptom in that category. Not the textbook description. Yours.

Be honest. Nobody is grading this. The single biggest predictor of cohort outcome is the accuracy of the day-one baseline. Overscoring "0" because you don't want to face it just delays the work. Score it the way it actually is today.

> ***Rating scale***: `0 – 1 – 2 – 3 – 4 – 5` *0 = no issue · 1 = rare · 2 = occasional · 3 = a few times a month · 4 = several times a week · 5 = daily and affecting my life*

Mind

01. Cognitive / brain fog

Mind is the destination of the 4M system — cognitive performance is the end-state we are optimizing for.

Rate today: `0 – 1 – 2 – 3 – 4 – 5` → My score: _____

Current symptom (word-finding, focus, foggy days per week — be specific):

---

02. Sleep

The brain cleans itself only during deep sleep. Poor sleep silently degrades hormones, weight, mood, and memory.

Rate today: `0 – 1 – 2 – 3 – 4 – 5` → My score: _____

Current symptom (trouble falling asleep, 3 AM waking, unrefreshed mornings — be specific):

---

03. Hearing, vision & dental

Uncorrected hearing loss is one of the largest modifiable dementia risk factors — and gum disease feeds the same inflammation that reaches the brain. The oral microbiome is the front door of the gut-brain axis.

Rate today: `0 – 1 – 2 – 3 – 4 – 5` → My score: _____

Current symptom (straining in conversation, reading strain, bleeding gums, overdue dental — be specific):

---

04. Mood

Depression and anxiety are among the most under-recognized contributors to cognitive decline — and among the most treatable.

Rate today: `0 – 1 – 2 – 3 – 4 – 5` → My score: _____

Current symptom (low mood, anxiety, short fuse — how often, how long — be specific):

---

05. Social connection

Social isolation carries a dementia risk on par with the big physical factors. Connection is a protocol, not a luxury.

Rate today: `0 – 1 – 2 – 3 – 4 – 5` → My score: _____

Current symptom (real conversations per week, who you actually see — be specific):

---

06. Mental challenge

The brain follows a ruthless use-it-or-lose-it principle. Novelty and difficulty are what keep the architecture live.

Rate today: `0 – 1 – 2 – 3 – 4 – 5` → My score: _____

Current symptom (what you learn or practice that is genuinely hard — be specific):

Muscle

07. Movement & strength

Muscle is the engine — and the single best-evidenced lever for both metabolic and cognitive protection.

Rate today: `0 – 1 – 2 – 3 – 4 – 5` → My score: _____

Current symptom (lifting sessions per week, daily steps, what you can no longer do — be specific):

---

08. Weight

Answer by how your clothes fit and how far you are from your ideal weight — no exact numbers needed.

Rate today: `0 – 1 – 2 – 3 – 4 – 5` → My score: _____

Current symptom (how clothes fit, pounds from target, belly fat — be specific):

---

09. Nutrition

Food quality is the daily input that drives microbiome diversity, blood sugar, and energy.

Rate today: `0 – 1 – 2 – 3 – 4 – 5` → My score: _____

Current symptom (takeout or processed meals per week, what breakfast actually is — be specific):

---

10. Pain / injury

Pain that blocks movement quietly costs you the muscle, sleep, and mood that protect the brain. It is fixable more often than people assume.

Rate today: `0 – 1 – 2 – 3 – 4 – 5` → My score: _____

Current symptom (where it hurts, what it stops you doing — be specific):

Mitigate

11. Blood pressure

Midlife blood pressure is one of the strongest predictors of late-life cognitive decline. Not knowing your numbers scores like a problem — because it is one.

Rate today: `0 – 1 – 2 – 3 – 4 – 5` → My score: _____

Current symptom (last reading and date — be specific):

---

12. Blood sugar

Alzheimer's is often called type 3 diabetes for a reason — the brain runs on glucose signaling, and insulin resistance degrades it.

Rate today: `0 – 1 – 2 – 3 – 4 – 5` → My score: _____

Current symptom (last A1c or fasting glucose and date — be specific):

---

13. LDL cholesterol

The arteries that feed the brain are the smallest in the body — they clog first. LDL is a brain number, not just a heart number.

Rate today: `0 – 1 – 2 – 3 – 4 – 5` → My score: _____

Current symptom (last LDL and date — be specific):

---

14. Smoking / nicotine

Smoking is the single most reversible vascular insult on this list — the brain benefits start within weeks of stopping.

Rate today: `0 – 1 – 2 – 3 – 4 – 5` → My score: _____

Current symptom (product and daily amount — be specific):

---

15. Alcohol

Alcohol disrupts gut, sleep, hormones, and cognition simultaneously. Cutting back affects every other pillar.

Rate today: `0 – 1 – 2 – 3 – 4 – 5` → My score: _____

Current symptom (drinks per week, drinks per sitting — honest — be specific):

---

16. Gut health

The gut-brain axis sits underneath nearly everything else — fix this first and other categories often improve on their own.

Rate today: `0 – 1 – 2 – 3 – 4 – 5` → My score: _____

Current symptom (bloating, food reactions, brain fog after meals, irregular stools — be specific):

---

17. Hormones

Testosterone, estrogen, progesterone, and cortisol balance underlie resilience — proper labs and a physician-led plan beat guessing.

Rate today: `0 – 1 – 2 – 3 – 4 – 5` → My score: _____

Current symptom (fatigue, drive, mood swings, recovery, muscle loss — and for women, cycle changes, hot flashes, night sweats — be specific):

---

18. Sexual function

Declining sexual function is an early-warning signal — the canary in the mine — for hormones, cardiovascular health, and cognitive trajectory. For men it shows up as ED; for women, often as the first sign of the perimenopausal transition. Not a verdict.

Rate today: `0 – 1 – 2 – 3 – 4 – 5` → My score: _____

Current symptom (desire, arousal, and for men morning erection frequency and hardness — be specific):

---

19. Environment

Light, air, water, and EMF compound over years. Lack of morning sunlight is the most overlooked driver of poor sleep and mood.

Rate today: `0 – 1 – 2 – 3 – 4 – 5` → My score: _____

Current symptom (morning sun minutes, water source, screen hours, air filtration — be specific):

Motivate

20. Purpose & accountability

Knowledge is not the bottleneck — accountability is. Everyone in the program names a specific face they are doing this for.

Rate today: `0 – 1 – 2 – 3 – 4 – 5` → My score: _____

Current symptom (who checks on you, what your written goal actually says — be specific):

Already diagnosed? (Yes / No)

Have you already been diagnosed with mild cognitive impairment, early dementia, Alzheimer's, or Parkinson's?

□ Yes □ No — a Yes makes the regenerative-medicine consult time-sensitive; book it in Week 1.

Personal physiology baselines

Six numbers the app charts, plus two only you can keep. Take them today, before Week 1. Then once a week: same day, same conditions. Log the first six on your app home screen so the chart builds.

Measure	Baseline	Wk 1	Wk 2	Wk 3	Wk 4
Body weight (lb)	_____	_____	_____	_____	_____
Waist at navel (in)	_____	_____	_____	_____	_____
Blood pressure (sys / dia)	_____	_____	_____	_____	_____
Resting pulse (bpm, AM)	_____	_____	_____	_____	_____
SpO_2 (%)	_____	_____	_____	_____	_____
Average sleep (hr/night, last 7 nights)	_____	_____	_____	_____	_____
Alcoholic drinks per week (honest)	_____	_____	_____	_____	_____

Date of today's baseline: _____

> ***Tear-out page — tape this to your bathroom mirror or screenshot for your phone lock-screen.***
>
> *The face I'm doing this for: _____*
>
> *My three six-month targets:*
>
> 1. _____
> 2. _____
> 3. _____
>
> The protocol that you run beats the protocol that was theoretically better. And the protocol that you run is the one you run for a specific named person.
>
> *Signed: _____ Date: _____*

Part 1 is complete when all four exercises are filled in by hand. Don't move on to Part 2 until the face has a name, the why has a signature, the three targets have numbers, and the baseline has a date. We'll build the protocol on top of that foundation in Part 2.

Part 2 — The 4M Framework

You have one operating system and four pillars that run on it.

This page is your anchor. Any time the protocol starts to feel like a pile of tasks instead of a coherent system, turn back here. Read it again. Find the task you're doing under one of the four Ms. Remember what it's serving.

Mind — The Destination

Mind is what we're protecting.

Not "longevity." Not "anti-aging." Not the body in the mirror. The person your spouse married. The parent your kids recognize. The operator who walks into a room and reads it. That person lives in roughly three pounds of tissue between your ears, and that tissue is what every other intervention in this logbook is serving.

When you forget what the point of any single behavior is, the answer is always the same: **the point is Mind.**

> Begin with the end in mind. The end is your mind.

Muscle — The Engine

> Strong body, sharp mind. Resistance training is neuroprotection.

Muscle is the substrate. The body is the delivery system. You do not get to keep the mind if you let the chassis rust.

Under Muscle: nutrition (the 30–40g protein-first rule), the eating window (9 AM – 6 PM), weight and visceral fat (the GLP-1 conversation if it applies), resistance training (2-3 sessions per week, compound lifts), strength preservation (sarcopenia reverses with deliberate work), and pain management (because pain is what stops training, and stopping training is what stops neuroprotection).

If you only ever ran one pillar, this is the one with the most measurable physical return.

Mitigate — Stop Hurting Yourself

> Stop hurting yourself first. Then add what works.

Mitigate is the pillar that removes the chronic insults silently degrading the system. Your gut. Your sleep. Your environment (light, air, water, EMF). Your hormones (and the canary that signals when they've started to drift — ED for men, the perimenopause/menopause cluster for women). The alcohol you've normalized. The substances you've made part of the routine.

Mitigate works in one direction at a time. You cannot meaningfully *add* a protocol while still actively running the input that broke you. The order is: stop the bleed, then start the rebuild.

This is the largest pillar in the cohort by surface area. Most of the audit categories live here. Most of the long-term cognitive return lives here too.

Motivate — What Sustains the Work

> What sustains the work — and what closes the loop.

Motivate is why you keep showing up on the third Tuesday in February when your shoulder is sore and the deal at work is falling apart and your daughter is going through something and nothing about the protocol sounds appealing.

Under Motivate: purpose (your written why from Part 1), identity (the person you're becoming, not the protocol you're trying), accountability (the face from Part 1 + the cohort), the structural enablers (a morning routine that runs without willpower, the friction-removal layer the My4MLife system is built around), and compliance (the architecture that makes running the protocol the path of least resistance).

The most rigorous protocol in the world fails if compliance fails. Motivate is the pillar that makes the other three sustainable.

The Loop

The four pillars are not a linear sequence. They are a recursive engine.

Each pass through the cycle strengthens what you came to protect. Mitigate removes the insults so Muscle has a clean substrate to work on. Muscle builds the body Mind depends on. Motivate sustains the compliance that makes Mitigate and Muscle keep running. And

then you arrive back at Mind, sharper than you started, because the work itself was neuroprotective.

Then you do it again. And again. The person who runs the loop longest wins.

Quick reference

Pillar	One-line job	Where it lives in your week
Mind	The destination — cognitive healthspan	Every reflection · Sunday review
Muscle	The engine — strength, nutrition, fuel	Lift days · protein-first meals
Mitigate	Eliminate the insulting behavior	Daily stack · sleep window · gut · hormones audit
Motivate	Sustain the work · close the loop	Weekly Zoom · accountability target · the why

---

> Begin with the end in mind. Don’t lose your identity and your dignity while you still have a choice.

Part 3 — Week 1: Foundation

3.1 — How Week 1 Works

This is the anchor week. Every week that follows sits on top of what you build here. If you skim it, you will pay for it in Week 3.

Week 1 is not about heroics. It is about laying down the architecture — the same six actions, repeated, across all four pillars — so that the deeper work in Weeks 2 through 4 has something to stand on. You are not trying to optimize. You are trying to make the floor non-negotiable.

Here is the entire week, in plain language:

- **Morning sun + walk.** Eight to fifteen minutes within the first hour of waking. Eyes toward the sun, sunglasses off, no food yet. Two times this week, minimum. Daily is better; the tracker has a box for it.
- **Protein-first breakfast.** Thirty to forty grams of lean protein at every fast-break, thirty minutes before any other macro enters your mouth. Every day. Your daily target is one gram per pound of your ideal body weight.
- **Eating window 9 AM to 6 PM.** Aim for every day; five of seven is the floor the app scores. A breakfast with the kids or grandkids doesn't break the program; walk thirty minutes after. First bite at nine or after. Last bite by six. Water, black coffee, and tea outside the window are fine. *Until you reach your ideal body weight, low body fat, and low visceral fat: train fasted in the morning — fasted morning walking in particular is a powerful accelerator. Break the fast after training with 30–40g of*

lean protein. If your workout ends around 10 AM, that's when the window opens. If you want to further regulate cortisol before sleep, experiment with moving your last bite earlier — around 4 or 5 PM — once the 9–6 window is consistent.

- **Lights out by 10:30 PM.** Daily. Backwards-plan your evening from this. Screens down by 9:45. Mag L-threonate at 9.
- **The daily stack.** Six products. See the card in §3.2. Anchored on the gut-barrier probiotic with your first meal. Non-negotiable.
- **Two strength sessions.** Compound lifts. Squat, hinge, push, pull, carry. Recovery-prioritized. Leave one rep in the tank.

Plus two touchpoints that hold the week together:

- **The weekly Zoom.** It is scheduled based on demand. If your schedule does not allow you to attend, the recording is available to you. The calendar moves you to Week 2 on its own either way. The Zoom is what keeps you from running it alone.
- **The Sunday reflection.** Four pages, one per pillar, at the back of this part. Do it Sunday evening with a cup of tea, not Monday morning in a panic.

There is no optional advanced layer this week. No cold plunges, no breathwork stacks, no zone-2 protocols. If you are tempted to add something, don't. The temptation to add is the same temptation that has made every previous attempt fail. *Stop hurting yourself first. Then add what works.*

Your job for the next seven days is to prove to yourself that you can do six simple things, repeatedly, while a stopwatch is running. That's it.

3.2 — The Week 1 Daily Stack Card

> ***Tear-out page — tape this inside your medicine cabinet, at eye level.***

My4MLife Daily Stack — Week 1

See my4mlife.com for the specific products we currently recommend.

When	Product	Dose	Why
With or right after your first meal	Gut-barrier probiotic (the gut-brain seal)	As labeled; see my4mlife.com/stack	Gut-brain seal. The foundation of every other pillar.
With breakfast (~9 AM)	Vitamin D + K2 + boron + astaxanthin stack	1 serving	D3 + K2 + boron + astaxanthin. Daily floor nutrients.
Training days: around the fasted lift · Non-training days: with breakfast	Creatine monohydrate	5 g	Training days: half a scoop of electrolytes + 5 g creatine in 16 oz water before the fasted lift, other half after. Non-training days: 5 g with breakfast.
Training days — half before the lift, half after	Electrolyte powder (zero-sugar, low-sodium; see my4mlife.com for the one we use)	1 scoop split in 16 oz water	Carries the creatine, holds hydration through a fasted lift, and blunts appetite if you want to push the fast further into the day.
With your highest-fat meal of the day	Omega-3 + ubiquinol CoQ10 softgel	2 softgels	EPA/DHA + ubiquinol CoQ10. Cardio-neuro. Take with fat for absorption.
30–60 min before lights out	Sleep-support formula	1 serving	Sleep onset + architecture.
Evening (around 9	Magnesium L-threonate	1.5–2 g	The only magnesium form that crosses into

When	Product	Dose	Why
PM)			the hippocampus.

> Same time, same place, every day. Stack lives where you can see it. If you have to hunt for it, you'll skip it.

3.3 — Daily Check-In Tracker

> ***One page per day. Fill it in at the end of the day, not the start.***

Monday — Week 1

Floor behavior	Done
Morning sun + walk (8–15 min)	☐
Protein-first breakfast (30–40g)	☐
Eating window closed by 6 PM	☐
Lights out by 10:30 PM	☐

Supplement	Done
Gut-barrier probiotic (with first meal)	☐
D3 + K2 stack (breakfast)	☐
Creatine 5 g (+ electrolytes around the lift on training days)	☐
Omega-3 + CoQ10 softgel (fat meal)	☐
Sleep-support formula (pre-bed)	☐
Magnesium L-threonate (evening)	☐

Morning energy (0–10): _ **Sleep quality last night** (0–10): _ **Mood** (0–10): _

Today's biggest win or biggest slip:

Tuesday — Week 1

Floor behavior	Done
Morning sun + walk (8–15 min)	☐
Protein-first breakfast (30–40g)	☐
Eating window closed by 6 PM	☐
Lights out by 10:30 PM	☐

Supplement	Done
Gut-barrier probiotic (with first meal)	☐
D3 + K2 stack (breakfast)	☐
Creatine 5 g (+ electrolytes around the lift on training days)	☐
Omega-3 + CoQ10 softgel (fat meal)	☐
Sleep-support formula (pre-bed)	☐
Magnesium L-threonate (evening)	☐

Morning energy (0–10): _ **Sleep quality last night** (0–10): _ **Mood** (0–10): _

Today's biggest win or biggest slip:

Wednesday — Week 1

Floor behavior	Done
Morning sun + walk (8–15 min)	☐
Protein-first breakfast (30–40g)	☐
Eating window closed by 6 PM	☐
Lights out by 10:30 PM	☐

Supplement	Done
Gut-barrier probiotic (with first meal)	☐
D3 + K2 stack (breakfast)	☐
Creatine 5 g (+ electrolytes around the lift on training days)	☐
Omega-3 + CoQ10 softgel (fat meal)	☐
Sleep-support formula (pre-bed)	☐
Magnesium L-threonate (evening)	☐

Morning energy (0–10): _ **Sleep quality last night** (0–10): _ **Mood** (0–10): _

Today's biggest win or biggest slip:

Thursday — Week 1

Floor behavior	Done
Morning sun + walk (8–15 min)	☐
Protein-first breakfast (30–40g)	☐
Eating window closed by 6 PM	☐
Lights out by 10:30 PM	☐

Supplement	Done
Gut-barrier probiotic (with first meal)	☐
D3 + K2 stack (breakfast)	☐
Creatine 5 g (+ electrolytes around the lift on training days)	☐
Omega-3 + CoQ10 softgel (fat meal)	☐
Sleep-support formula (pre-bed)	☐
Magnesium L-threonate (evening)	☐

Morning energy (0–10): _ **Sleep quality last night** (0–10): _ **Mood** (0–10): _

Today's biggest win or biggest slip:

Friday — Week 1

Floor behavior	Done
Morning sun + walk (8–15 min)	☐
Protein-first breakfast (30–40g)	☐
Eating window closed by 6 PM	☐
Lights out by 10:30 PM	☐

Supplement	Done
Gut-barrier probiotic (with first meal)	☐
D3 + K2 stack (breakfast)	☐
Creatine 5 g (+ electrolytes around the lift on training days)	☐
Omega-3 + CoQ10 softgel (fat meal)	☐
Sleep-support formula (pre-bed)	☐
Magnesium L-threonate (evening)	☐

Morning energy (0–10): _ **Sleep quality last night** (0–10): _ **Mood** (0–10): _

Today's biggest win or biggest slip:

Saturday — Week 1

Floor behavior	Done
Morning sun + walk (8–15 min)	☐
Protein-first breakfast (30–40g)	☐
Eating window closed by 6 PM	☐
Lights out by 10:30 PM	☐

Supplement	Done
Gut-barrier probiotic (with first meal)	☐
D3 + K2 stack (breakfast)	☐
Creatine 5 g (+ electrolytes around the lift on training days)	☐
Omega-3 + CoQ10 softgel (fat meal)	☐
Sleep-support formula (pre-bed)	☐
Magnesium L-threonate (evening)	☐

Morning energy (0–10): _ **Sleep quality last night** (0–10): _ **Mood** (0–10): _

Today's biggest win or biggest slip:

Sunday — Week 1

Floor behavior	Done
Morning sun + walk (8–15 min)	☐
Protein-first breakfast (30–40g)	☐
Eating window closed by 6 PM	☐
Lights out by 10:30 PM	☐

Supplement	Done
Gut-barrier probiotic (with first meal)	☐
D3 + K2 stack (breakfast)	☐
Creatine 5 g (+ electrolytes around the lift on training days)	☐
Omega-3 + CoQ10 softgel (fat meal)	☐
Sleep-support formula (pre-bed)	☐
Magnesium L-threonate (evening)	☐

Morning energy (0–10): _ **Sleep quality last night** (0–10): _ **Mood** (0–10): _

Today's biggest win or biggest slip:

3.4 — Strength Session Log

> Squat / hinge / push / pull / carry. Two to three sessions per week. Compound lifts, recovery-prioritized, leave one rep in the tank.

Session 1

Date: _____ **Start time:** _____ **End time:** _____

Movement	Sets × Reps × Weight	Notes

How I felt going in (0–10): _____ How I felt walking out (0–10): _____

One thing to change next session: _____

Session 2

Date: _____ Start time: _____ End time: _____

Movement	Sets × Reps × Weight	Notes

How I felt going in (0–10): _____ How I felt walking out (0–10): _____

One thing to change next session: _____

3.5 — Weekly Zoom Prep Page

> Show up to the Zoom even on the weeks you fell off. Especially those weeks.

Fill this in before the call. Bring it with you.

1. What went well this week:

2. What slipped — and what I think caused it:

3. One question I want to ask Dr. TJ:

4. One thing the cohort should know about my week:

3.6 — End-of-Week 4M Reflection

Do this Sunday evening. Pen, not phone. Slowly.

Mind

Mind is the destination. This week's Mind work was about anchoring your circadian rhythm — morning sun, lights out by 10:30, magnesium L-threonate at night — and giving your brain the substrates it needs to do its job. The 4M program protects cognitive healthspan. Mind is what you're trying to keep.

1. What did my mornings feel like this week compared to a typical week?

2. When during the day did I feel sharpest? When did I feel foggiest?

3. Did I notice any difference in sleep onset or sleep depth on the nights I followed the protocol vs. the nights I slipped?

How did Mind go this week? 0 — 1 — 2 — 3 — 4 — 5 — 6 — 7 — 8 — 9 — 10

One adjustment I'm committing to for next week:

Muscle

Strong body, sharp mind. Resistance training is neuroprotection. This week you ran two strength sessions, opened every feeding window with thirty to forty grams of lean protein, and added creatine to your morning. Muscle is the engine — it pulls glucose out of your

blood, releases the myokines that talk to your brain, and is the single most modifiable predictor of how you'll age.

1. How did my two strength sessions actually go — honestly?

2. Did I hit 30–40g of protein at first bite, or did I drift into carbs first?

3. Where in the day did my energy hold up best? Where did it crash?

How did Muscle go this week? 0 — 1 — 2 — 3 — 4 — 5 — 6 — 7 — 8 — 9 — 10

One adjustment I'm committing to for next week:

Mitigate

Stop hurting yourself first. Then add what works. This week's Mitigate work centered on the gut — the gut-barrier probiotic (the gut-brain seal) every day with your first meal — and on closing your eating window by 6 PM so your gut, liver, and pancreas got real rest. Mitigate is the pillar where you remove the chronic insults driving neuroinflammation. You can't out-supplement a leaking gut or a 10 PM snack.

1. What insulting behavior did I notice myself doing this week — and did I stop it?

2. How did my gut actually feel by Friday compared to Monday?

3. What's the single insult I'm not yet ready to give up — and why?

How did Mitigate go this week? 0 — 1 — 2 — 3 — 4 — 5 — 6 — 7 — 8 — 9 — 10

One adjustment I'm committing to for next week:

Motivate

What sustains the work — and what closes the loop. Motivate is the pillar nobody pays enough attention to until they fall off. This week, Motivate looked like showing up to the weekly Zoom and naming the face you're doing this for. Identity and accountability are not soft skills. They are the structural enablers that determine whether you do this for seven days or fifty-two weeks.

1. Whose face did I picture this week when I didn't want to do the work?

2. Did I show up to the weekly Zoom — live or recording? If not, why?

3. Who in my life this week did I tell — out loud — that I'm doing this?

How did Motivate go this week? 0 — 1 — 2 — 3 — 4 — 5 — 6 — 7 — 8 — 9 — 10

One adjustment I'm committing to for next week:

3.7 — The Week 1 Adherence Scorecard

> ***Tear-out page — tape this to your bathroom mirror or screenshot for your phone lock-screen.***

My4MLife — Week 1 Adherence

Action	M	T	W	T	F	S	S
Morning sun + walk	☐	☐	☐	☐	☐	☐	☐
Protein-first breakfast (30–40g)	☐	☐	☐	☐	☐	☐	☐
Eating window closed by 6 PM	☐	☐	☐	☐	☐	☐	☐
Lights out by 10:30 PM	☐	☐	☐	☐	☐	☐	☐
Gut-barrier probiotic (first meal)	☐	☐	☐	☐	☐	☐	☐
D3 + K2 stack + creatine (+ electrolytes around the lift on training days)	☐	☐	☐	☐	☐	☐	☐
Omega-3 + CoQ10 softgel (with fat meal)	☐	☐	☐	☐	☐	☐	☐
Sleep-support formula (pre-bed)	☐	☐	☐	☐	☐	☐	☐
Magnesium L-threonate (evening)	☐	☐	☐	☐	☐	☐	☐
Lift session (2–3/wk)	☐	☐	☐	☐	☐	☐	☐
Lift session (2–3/wk)	☐	☐	☐	☐	☐	☐	☐
Weekly Zoom attended			☐				
Sunday reflection done							☐

Weekly target: _ / 79

Protocol Score this week: _____ (the ring on your app home screen; higher is better)

> Begin with the end in mind.
>
> Don’t lose your identity and your dignity while you still have a choice.

Part 4 — Week 2: Add the Lift

> Strong body, sharp mind. Resistance training is neuroprotection.

4.1 — How Week 2 works

Week 1 built the architecture. Week 2 starts the engine.

You kept everything from Week 1 — morning sun and walk, 30–40g protein-first, 9 AM to 6 PM eating window, the gut-barrier probiotic / D3+K2 stack / creatine / omega-3+CoQ10 softgel / sleep-support formula / Mag L-threonate stack, lights out by 10:30. None of that goes away. It's the foundation. We don't trade Week 1 for Week 2 — we stack Week 2 on top.

What we add this week is the lift.

Here's what most people in your decade don't understand: by midlife, sarcopenia has already started. You've been losing 1-2% of lean muscle per year since your mid-30s, and it accelerates from here. You don't see it because the scale doesn't move much — fat fills in where muscle leaves. The mirror lies politely. The bloodwork doesn't.

Sarcopenia is not a falls-and-fractures problem. It's a cognitive risk factor. Compound resistance training releases myokines — irisin, BDNF-adjacent signaling molecules, cathepsin B — that cross the blood-brain barrier and stimulate neurogenesis in the

hippocampus. The same hippocampus that shrinks first in Alzheimer's. When I say *resistance training is neuroprotection*, I mean it literally. The squat is a memory drug. The deadlift is a mood drug. The carry is a cognition drug.

Three sessions this week. A / B / A pattern. Compound lifts only. Leave one rep in the tank — recovery is where adaptation happens, not in the gym.

We're also tightening sleep architecture. The 60 minutes before bed are the single highest-leverage window most people waste. You're going to install a real wind-down this week.

Same daily check-in. Same weekly Zoom. Same end-of-week 4M reflection. New trackers for the lift and the wind-down.

4.2 — The Week 2 Strength Protocol

Pattern: Mon — Session A · Wed — Session B · Fri — Session A (Or Tue / Thu / Sat if your week runs that way. Keep at least one full rest day between sessions.)

Rule: Leave one rep in the tank on every set. If 8 reps was hard but you could have done 9, that's the right weight. If you grind the last rep, drop the weight 10% next set.

Progression: Add weight or one rep per movement, week to week. Slow is fine. Consistent beats heroic.

Warmup (every session — 5 to 8 minutes)

No static stretching cold. Move first, stretch later (or not at all).

Movement	Duration
Thoracic rotations (open-book, each side)	60 sec
Hip circles + 90/90 transitions	60 sec
Bodyweight squat to stand (slow)	10 reps
Light row + light press (band or empty bar)	8 reps each
Loaded carry — light, just to wake up the grip	30 sec

Session A — Squat + Push + Carry

Movement	Sets × Reps	Notes
Squat variation (back, front, goblet, or safety bar)	3 × 6-8	Pick the one your knees and back tolerate
Push (bench, DB press, or strict overhead)	3 × 8-10	Pause at chest, control the down
Loaded carry (farmer or suitcase)	3 × 30 sec	Heavy enough that the last 5 sec is honest work

Session B — Hinge + Pull + Carry

Movement	Sets × Reps	Notes
Hinge (trap-bar deadlift, RDL, or kettlebell swing)	3 × 6-8	Spine neutral. No ego.
Pull (chest-supported row, one-arm DB row, or lat pulldown)	3 × 8-10	Squeeze the shoulder blade
Loaded carry (farmer or front-rack)	3 × 30 sec	Switch hand position from Session A

Cooldown (every session — 3 to 5 minutes)

Walk. Outside if you can. Don't sit down sweaty. Let the heart rate come down on its feet.

Pre-workout (optional): Creatine + L-citrulline + beetroot + electrolyte blend (pre-workout), 20-30 min before training, with water. See my4mlife.com for the specific product we currently recommend. **Electrolytes on training days:** half a scoop of electrolyte powder (zero-sugar, low-sodium; see my4mlife.com for the one we use) with 5 g creatine in 16 oz water before the fasted lift, the other half after. It also blunts appetite if you want to push the fast further into the day. Non-training days: creatine with breakfast as before. **Post-workout:** 30–40g protein within 60 minutes. This is your protein-first feeding window opener — train fasted, then break the fast with protein after. If your workout ends around 10 AM, that's when the window opens.

> Recovery is where adaptation happens. Leave one rep in the tank.

4.3 — Strength Session Log

Print one of these for each session. Three per week.

Session ____ · Date: _____ · A / B (circle one)

Warmup completed: □ Yes □ Partial □ Skipped

Movement	Set 1 (reps × wt)	Set 2 (reps × wt)	Set 3 (reps × wt)	RPE (0-10)
_____	__ × _____	__ × _____	__ × _____	_____
_____	__ × _____	__ × _____	__ × _____	_____
Carry: _____	__ sec × _____	__ sec × _____	__ sec × _____	_____

RPE key: 5 = easy · 7 = challenging, 3 reps left · 8 = hard, 2 reps left · 9 = grinding, 1 rep left · 10 = failure

Cooldown walk completed: □ Yes □ Skipped

Post-session energy (0-10): _ Pain or red flags? □ None □ Minor — describe: _____ □ Stop and assess

Next session adjustment (one line):

Session ____ · Date: _____ · A / B (circle one)

Warmup completed: □ Yes □ Partial □ Skipped

Movement	Set 1 (reps × wt)	Set 2 (reps × wt)	Set 3 (reps × wt)	RPE (0-10)
_____	__ × _____	__ × _____	__ × _____	_____
_____	__ × _____	__ × _____	__ × _____	_____
Carry: _____	__ sec × _____	__ sec × _____	__ sec × _____	_____

Cooldown walk completed: □ Yes □ Skipped

Post-session energy (0-10): _ Pain or red flags? □ None □ Minor — describe: _____ □ Stop and assess

Next session adjustment (one line):

Session ____ · Date: _____ · A / B (circle one)

Warmup completed: □ Yes □ Partial □ Skipped

Movement	Set 1 (reps × wt)	Set 2 (reps × wt)	Set 3 (reps × wt)	RPE (0-10)
_____	__ × _____	__ × _____	__ × _____	_____
_____	__ × _____	__ × _____	__ × _____	_____
Carry: _____	__ sec × _____	__ sec × _____	__ sec × _____	_____

Cooldown walk completed: □ Yes □ Skipped

Post-session energy (0-10): _ Pain or red flags? □ None □ Minor — describe: _____ □ Stop and assess

Next session adjustment (one line):

4.4 — Sleep Architecture Tighten-Up

Lights out is still 10:30. What changes this week is the 60 minutes before it.

Most people sabotage sleep in the last hour of the day and then blame the supplement. Fix the hour first. The supplement does the rest.

The 9:30 PM wind-down — six rules

1. **No screens.** Phone down, TV off, laptop closed. If something is unavoidable, red-spectrum glasses only. Blue light past 9 PM suppresses melatonin for hours.
2. **No food.** Eating window closed at 6 PM. Late calories spike insulin and core temperature — both wreck deep sleep.
3. **No work, no email, no news.** The cortisol spike from one unread message lasts longer than the message takes to read.
4. **Dim ambient light.** One warm lamp. Overheads off. Your brain reads bright overhead light as noon.
5. **Cool the room.** 65-68°F. Core temperature drop is the signal that initiates sleep.
6. **Take the stack.** Mag L-threonate (1.5-2g) + sleep-support formula, 30-60 min before lights out. Water, not a full glass — you don't want a 3 AM bathroom trip.

Wind-Down Tracker — Week 2

Each night, check what you actually hit. Rate "felt rested" the next morning.

Night	No screens	No food	No work	Dim light	Cool room	Stack taken	Felt rested 0-10 (next AM)
Mon	□	□	□	□	□	□	_____
Tue	□	□	□	□	□	□	_____
Wed	□	□	□	□	□	□	_____
Thu	□	□	□	□	□	□	_____
Fri	□	□	□	□	□	□	_____
Sat	□	□	□	□	□	□	_____
Sun	□	□	□	□	□	□	_____

Pattern I notice (one line): _____

4.5 — Daily Check-In Tracker

One page per day. Print seven. Lift boxes only activate on lift days.

Day: □ Mon □ Tue □ Wed □ Thu □ Fri □ Sat □ Sun · Date: _____

Morning - □ Out of bed by 6:30 - □ Sun + walk (8-15 min, no sunglasses) - □ Gut-barrier probiotic with first meal - □ 30–40g protein-first to open the eating window (after training if you trained fasted) - □ D3 + K2 stack with breakfast; creatine with breakfast, or around the lift on training days - □ Omega-3 + CoQ10 softgel with largest fat-containing meal

Strength (if lift day) - □ Session A / B (circle) completed - □ Log filled in (§4.3) - □ Post-workout 30–40g protein (this is your feeding window opener)

Evening - □ Eating window closed by 6 PM - □ 9:30 PM wind-down started - □ Mag L-threonate + sleep-support formula - □ Lights out by 10:30

Daily ratings (0-10) - Energy: _____ Mood: _____ Focus: _____ Hunger control: _____

One sentence on today: _____

4.6 — Weekly Zoom Prep Page

Bring this filled in. Five minutes before the call.

Wins from this week so far:

Where I fell off:

---

1. One thing about lifting that surprised me this week:

---

2. One thing about sleep that surprised me:

---

3. One question for Dr. TJ about progression:

The face I'm doing this for, restated in one sentence:

4.7 — End-of-Week 4M Reflection

Sit down Sunday evening. One page per pillar. Muscle gets the heaviest weight this week.

Mind

This week, my best cognitive hour was: _____

My worst: _____

One mental-performance pattern I noticed:

On a 0-10 scale, how present was I with the people who matter? _____

Muscle *(heaviest section this week)*

1. What did 3 sessions feel like — physically and mentally?

2. What's the biggest tension between "feel like lifting" and "show up to lift"?

3. What's one piece of friction I can remove from the lift architecture next week? (Examples: gym bag pre-packed, clothes laid out, calendar block, training partner, shorter drive.)

Protein-first hits this week: _____ / 7 fast-breaks

Soreness pattern (where, how long): _____

Energy after lifts vs. before (0-10 shift): Before _____ → After _____

Mitigate

Eating window closed by 6 PM: _____ / 7 nights

9:30 PM wind-down ran clean: _____ / 7 nights

Best "felt rested" score this week: _____ **Worst:** _____

One insulting behavior I caught myself doing — and stopped:

Motivate

Did I stay connected to the face I'm doing this for? □ Yes □ Drifted □ Lost it

One person who noticed something different about me this week:

One identity statement that's getting truer:

Complete the line that's yours.

> I am a man who _____.
> I am a woman who _____.

What I want Week 3 to look like, in one sentence:

4.8 — Week 2 Adherence Scorecard

Tear-out page — tape this to your bathroom mirror or screenshot for your phone lock-screen.

Week 2 — The Lift Goes In

Behavior	M	T	W	T	F	S	S
Sun + walk	☐	☐	☐	☐	☐	☐	☐
Protein-first (30–40g)	☐	☐	☐	☐	☐	☐	☐
9-6 eating window	☐	☐	☐	☐	☐	☐	☐
Daily stack (gut-barrier probiotic + D3+K2 stack + creatine + omega-3+CoQ10 softgel)	☐	☐	☐	☐	☐	☐	☐
9:30 wind-down	☐	☐	☐	☐	☐	☐	☐
Mag L-threonate + sleep-support formula	☐	☐	☐	☐	☐	☐	☐
Lights out by 10:30	☐	☐	☐	☐	☐	☐	☐

Strength sessions this week:

☐ Session 1 (A) ☐ Session 2 (B) ☐ Session 3 (A)

Week 2 score: _____ / 52 boxes

Protocol Score this week: _____ (the ring on your app home screen; higher is better)

> Begin with the end in mind.
> Don't lose your identity and your dignity while you still have a choice.
> Strong body, sharp mind. Resistance training is neuroprotection.

Part 5 — Week 3: Hormones and the Canaries

5.1 — How Week 3 works

> ***How to run this week:*** *men run §5.2 and §5.3 (the ED canary). Women run §5.2b and §5.3b (the perimenopause canary). Everyone runs §5.4 onward — the decision tree, the daily tracker, the Zoom prep, and the reflection are shared.*

Two bodies, two alarms, the same four fires underneath: hormones, cardiovascular, cognitive, and quality of life. A man's alarm is loud and humiliating, so he eventually acts on it. A woman's is quiet and easy to blame on her life instead of her physiology, so she gets told to manage her stress for six years. Both birds are singing about the same gas in the same shaft. This week you find out what yours has been saying.

For men — the ED canary

Most men I see in their forties and fifties have never had an honest conversation about what's happening below the belt — and they've certainly never connected it to what's happening above the neck. That ends this week.

Erectile dysfunction is not a separate problem. It is the first visible warning across four systems at once: hormones, cardiovascular, cognitive, and quality of life. It shows up

early for a mechanical reason most men have never been told. Penile arteries are 1-2 millimeters wide. Coronary arteries are 3-4 millimeters. Endothelial dysfunction — the same arterial dysfunction that becomes a heart attack — shows up in the small arteries first, often three to five years before any cardiologist will see it on a stress test. By the time the cardiologist catches it, you've had three to five years of warning you ignored.

That's the canary. ED is the warning. The mine is your whole vascular and hormonal system, and your brain runs on the same plumbing.

This week we do not start a new protocol. Everything from Week 1 and Week 2 stays in place — the protein-first rule, the 9 AM-6 PM eating window (train fasted, break the fast with 30–40g protein after), morning sun, the gut-barrier probiotic, the D3+K2 stack, creatine, the omega-3+CoQ10 softgel, the sleep-support formula, magnesium L-threonate, the lifts, the lights-out at 10:30. None of that changes.

For women — the perimenopause canary

Perimenopause is the first readable signal a woman's body sends, and it is almost always read as something else. Sleep that stopped holding. A mood you do not recognize as yours. Fog that makes you write things down you never had to write down. Weight that relocates to the middle on the same food and the same training. Six years of being told to manage your stress.

Estrogen, progesterone, sleep architecture, and metabolism do not move one at a time. They move together, and they move through the brain. That makes this a cognitive event, not only a comfort one — which is exactly why it belongs in a logbook about protecting your mind rather than in a pamphlet about hot flashes.

Your body sends this signal as a cluster rather than a single event: sleep that breaks at two or three in the morning, word-finding lapses that frighten you more than you admit, a shorter fuse where enthusiasm used to be, weight that relocates to the middle on the same calories, and intimacy that quietly gets complicated. Five signals, same four fires. Perimenopause often begins in the early forties while cycles are still perfectly regular, which is why the standard screening question — *are your periods still coming?* — misses it for years.

Estrogen is not a reproductive hormone that happens to have effects elsewhere. It is a neurological hormone, with receptors dense in the hippocampus, the prefrontal cortex, and the hypothalamus. It supports cerebral blood flow through the same endothelial nitric-oxide pathway the ED canary reports on. When it falls — over a compressed handful of years, not a slow decades-long drift — the brain goes through a real metabolic transition. The fog is not imagined. It has a mechanism.

Timing appears to matter. Beginning a hormone conversation near the transition, with a vascular system that is still healthy, is a different proposition than beginning a decade after the last period. Nobody honest will tell you hormone therapy prevents dementia. But the window is real enough that *waiting to see how it goes* is itself a decision with consequences — one to make deliberately, with a clinician, not by default.

What both tracks do

What changes is that you are going to audit yourself honestly across ten markers you have probably never rated in writing, walk through a four-domain canary worksheet, and make one decision: do you engage the Rx consult layer this month, or do you stay OTC and reassess in 90 days?

This is a decision-week, not a new-protocol week. Show up to the page honest. Nobody else reads this.

> Eliminate the insulting behavior. Then add what works.

5.2 — The Hormone + Sexual Function Self-Audit

Men's track. Women: skip to §5.2b.

Rate each item 0 to 10. Then write one or two lines describing the current symptom in plain language — how it actually shows up in your week.

If the honest number embarrasses you, that is the number to write. The whole point of this page is that nobody is grading it but you.

---

1. Morning erections. *(0 = absent always, 10 = present most mornings, full quality)*

`0 — 1 — 2 — 3 — 4 — 5 — 6 — 7 — 8 — 9 — 10`

Current reality, in plain language:

---

2. Libido — desire for intimacy compared to 5 years ago. *(0 = gone, 5 = same as 5 years ago, 10 = stronger than 5 years ago)*

`0 — 1 — 2 — 3 — 4 — 5 — 6 — 7 — 8 — 9 — 10`

Current reality:

---

3. Erection quality during intimacy. *(0 = cannot achieve or maintain, 10 = reliable, full)*

`0 — 1 — 2 — 3 — 4 — 5 — 6 — 7 — 8 — 9 — 10`

Current reality:

---

4. Recovery after exertion. How many days do you need to feel normal after a hard workout, a long travel day, or a physical weekend?

Days needed: _____

(0 = days to feel normal, 10 = next day)

`0 — 1 — 2 — 3 — 4 — 5 — 6 — 7 — 8 — 9 — 10`

Five years ago this number was: _____

What's different now:

5. Energy at 3 PM. *(0 = crashed, need caffeine or a nap, 10 = sustained, sharp)*

`0 — 1 — 2 — 3 — 4 — 5 — 6 — 7 — 8 — 9 — 10`

Current reality:

6. Mood stability. *(0 = flat or reactive, 10 = grounded, even)*

`0 — 1 — 2 — 3 — 4 — 5 — 6 — 7 — 8 — 9 — 10`

Where it shows up most — work, marriage, kids, driving:

7. Muscle gain response to lifting. *(0 = absent — I lift and nothing changes, 10 = visible response within weeks)*

`0 — 1 — 2 — 3 — 4 — 5 — 6 — 7 — 8 — 9 — 10`

Current reality:

8. Fat distribution — visceral creep around the midsection. *(0 = significant new midsection fat in last 5 years, 10 = unchanged or leaner)*

`0 — 1 — 2 — 3 — 4 — 5 — 6 — 7 — 8 — 9 — 10`

Waist size 5 years ago: _____ Waist size today: _____

9. Sleep quality. *(0 = restless, fragmented, non-restorative, 10 = wake refreshed)*

`0 — 1 — 2 — 3 — 4 — 5 — 6 — 7 — 8 — 9 — 10`

Current reality:

10. Cognitive sharpness compared to 5 years ago. *(0 = noticeably duller, 5 = same, 10 = sharper)*

`0 — 1 — 2 — 3 — 4 — 5 — 6 — 7 — 8 — 9 — 10`

Where the dulling shows up — names, words, calendar, decision fatigue:

Total honest score (sum of all 10): _____ / 100

A score below 60 is not a verdict. It is information. It means several of these systems are talking to you and you have not been listening.

5.3 — The 4-Domain ED Canary Worksheet

Men's track. Women: skip to §5.3b.

Walk each domain. Answer in writing. End each domain with a single decision check.

Domain 1 — Hormones

Have you had a full hormone panel in the last 12 months? □ Yes □ No □ Never

Already on testosterone? □ Yes □ No Since: _____

Last total testosterone: _____ ng/dL Date: _____

Last free testosterone: _____ SHBG: _____

Last estradiol (E2): _____ Last DHEA-S: _____

If you wrote "never" or "I don't know" on any line above, circle it. That is the gap.

Is this domain a current concern? □ Yes □ Mild □ No

Domain 2 — Cardiovascular

Resting blood pressure (last reading): _____ / _____

Last lipid panel — Total chol: _____ LDL: _____ HDL: _____ Triglycerides: _____

Last fasting insulin: _____ Last HbA1c: _____

Do you snore? □ Yes □ No Witnessed apnea? □ Yes □ No Daytime sleepiness? □ Yes □ No

ED canary cross-check: the small arteries down there are 1-2mm. The coronaries are 3-4mm. If the small ones are struggling, the big ones are next.

Is this domain a current concern? □ Yes □ Mild □ No

Domain 3 — Cognitive

Brain fog frequency: □ Rare □ Weekly □ Most days □ Constant

Word recall delay (the word is on the tip of your tongue): □ Rare □ Weekly □ Daily □ Multiple times daily

Mood lability — irritability, flat affect, short fuse with people you love: □ Rare □ Weekly □ Most days

What your spouse or closest person has commented on in the last year:

Is this domain a current concern? □ Yes □ Mild □ No

Domain 4 — Quality of Life

Intimacy frequency now: _____ / month Five years ago: _____ / month

Confidence in your own body (0-10): _____

Avoidance behaviors — name them honestly. Skipping intimacy. Avoiding the mirror. Avoiding the scale. Avoiding the doctor. Avoiding the photo.

Is this domain a current concern? □ Yes □ Mild □ No

Domain tally:

Yes: _____ Mild: _____ No: _____

The one decision this month. You either engage the Rx consult layer now — the **Testosterone & ED lane**, booked through a free call with the care coordinator at my4mlife.com/consult — or you stay OTC and reassess with this same worksheet in 90 days. Both are legitimate. Defaulting is not.

□ Engaging the Rx consult layer this month — Testosterone & ED lane. Date booked: _____

□ Staying OTC. Reassessment date (90 days out): _____

Then run §5.4 with everyone else.

5.2b — The Other Canary: Perimenopause / Menopause Self-Audit

Women's track. Men: you ran §5.2.

Rate each item 0 to 10. Then write one or two lines describing how it actually shows up in your week, in plain language.

You have probably been told by more than one office that your labs are fine and you are just stressed. This page is where you write down what you actually notice, in your own handwriting, so that it stops being a feeling and becomes data.

If the honest number embarrasses you, that is the number to write. Nobody is grading it but you.

---

1. Sleep continuity — waking at 2 or 3 AM. *(0 = wake most nights and lie there, 10 = sleep through)*

`0 — 1 — 2 — 3 — 4 — 5 — 6 — 7 — 8 — 9 — 10`

How often, and how long are you awake:

---

2. Night sweats or hot flashes. *(0 = daily and disruptive, 10 = none)*

`0 — 1 — 2 — 3 — 4 — 5 — 6 — 7 — 8 — 9 — 10`

When they hit — night, afternoon, under stress, after a drink:

---

3. Word-finding and recall compared to 5 years ago. *(0 = noticeably duller, 5 = same, 10 = sharper)*

`0 — 1 — 2 — 3 — 4 — 5 — 6 — 7 — 8 — 9 — 10`

Where the dulling shows up — names, words mid-sentence, walking into a room:

4. Mood — fuse length and flatness. *(0 = short fuse, flat, anxious in the body before the thoughts; 10 = grounded, even)*

`0 — 1 — 2 — 3 — 4 — 5 — 6 — 7 — 8 — 9 — 10`

Where it shows up most — work, marriage, kids, driving:

5. Energy at 3 PM. *(0 = crashed, need caffeine or a nap, 10 = sustained, sharp)*

`0 — 1 — 2 — 3 — 4 — 5 — 6 — 7 — 8 — 9 — 10`

Current reality:

6. Body composition — where the weight sits. *(0 = significant new midsection fat in last 5 years, 10 = unchanged or leaner)*

`0 — 1 — 2 — 3 — 4 — 5 — 6 — 7 — 8 — 9 — 10`

Waist size 5 years ago: _____ Waist size today: _____

Did anything about how you eat or move actually change? □ Yes □ No

---

7. Muscle and strength response to training. *(0 = absent — I train and nothing changes, 10 = visible response within weeks)*

`0 — 1 — 2 — 3 — 4 — 5 — 6 — 7 — 8 — 9 — 10`

Current reality:

---

8. Libido and comfort with intimacy compared to 5 years ago. *(0 = gone or physically uncomfortable, 5 = same as 5 years ago, 10 = stronger)*

`0 — 1 — 2 — 3 — 4 — 5 — 6 — 7 — 8 — 9 — 10`

Current reality — including anything physical you have never raised with a clinician:

---

9. Cycle changes (if still cycling) / time since last period.

Still cycling? □ Yes □ Irregular □ No

If still cycling — what has changed in length, flow, or PMS in the last 2 years:

If not — date of last period: _____

Note: perimenopause frequently begins while cycles are still perfectly regular. A normal cycle does not rule this out.

(0 = markedly changed, 10 = unchanged. Past your last period? Rate how settled things are. No cycle to read — hysterectomy, ablation, a hormonal IUD? Note it above and score this one 5.)

`0 — 1 — 2 — 3 — 4 — 5 — 6 — 7 — 8 — 9 — 10`

---

10. Joint aches, skin, hair, and urinary or genitourinary changes. *(0 = multiple new symptoms, 10 = none)*

`0 — 1 — 2 — 3 — 4 — 5 — 6 — 7 — 8 — 9 — 10`

Which ones, specifically:

Total honest score (sum of all 10): _____ / 100

A score below 60 is not a verdict. It is information. It means several of these systems are talking to you and nobody — including you — has been listening as one signal.

5.3b — The 4-Domain Menopause Canary Worksheet

Women's track. Men: you ran §5.3.

Same four fires, same structure. Walk each domain. Answer in writing. End each domain with a single decision check.

Domain 1 — Hormones

Have you had a full hormone panel in the last 12 months? □ Yes □ No □ Never

Already on hormone therapy? □ Yes □ No Since: _____

Last estradiol (E2): _____ Last progesterone: _____ Last FSH: _____

Last total testosterone: _____ Free testosterone: _____ SHBG: _____

Last TSH: _____ AM cortisol: _____ DHEA-S: _____

One number in isolation tells you nothing here. Estradiol swings erratically in perimenopause — that is the point, not the exception.

If you wrote "never" or "I don't know" on any line above, circle it. That is the gap.

Is this domain a current concern? □ Yes □ Mild □ No

Domain 2 — Cardiovascular

Resting blood pressure (last reading): _____ / _____

Last lipid panel — Total chol: _____ LDL: _____ HDL: _____ Triglycerides: _____

Last fasting glucose: _____ Last HbA1c: _____ Last hs-CRP: _____

Sleep-apnea screen — exhausted after eight hours in bed? □ Yes □ No Morning headache? □ Yes □ No Insomnia treated as anxiety? □ Yes □ No

Apnea is badly underdiagnosed in women because it presents as insomnia, fatigue, headache, and low mood rather than loud snoring — and risk rises sharply after menopause. Cardiovascular disease is the leading cause of death in women; the risk curve climbs after the transition until it converges with men's.

Is this domain a current concern? □ Yes □ Mild □ No

Domain 3 — Cognitive

Brain fog frequency: □ Rare □ Weekly □ Most days □ Constant

Word recall delay (the word is on the tip of your tongue): □ Rare □ Weekly □ Daily □ Multiple times daily

Have you privately wondered whether this is early dementia? □ Yes □ No

Nearly every woman who rates this honestly has. It is worth writing down rather than carrying.

Are you writing things down that you never had to write down before? □ Yes □ No

What your spouse, partner, or closest person has commented on in the last year:

Is this domain a current concern? □ Yes □ Mild □ No

Domain 4 — Quality of Life

Confidence in your own competence at work or at home (0-10): _____

Intimacy frequency now: _____ / month Five years ago: _____ / month

Has physical discomfort — not desire — become part of why? □ Yes □ No □ Both

The true sentence most people never say out loud is *my body is not doing what it used to do and I don't know how to bring it up*. Write your version of it here:

Avoidance behaviors — name them honestly. Skipping intimacy. Skipping the annual. Skipping the mirror, the scale, the photo. Managing everyone else's health and deferring your own.

Is this domain a current concern? □ Yes □ Mild □ No

Domain tally:

Yes: _____ Mild: _____ No: _____

The one decision this month. You either engage the Rx consult layer now — the **Menopause & HRT lane**, booked through a free call with the care coordinator at my4mlife.com/consult — or you stay OTC and reassess with this same worksheet in 90 days. Both are legitimate. Defaulting is not.

□ Engaging the Rx consult layer this month — Menopause & HRT lane. Date booked: _____

□ Staying OTC. Reassessment date (90 days out): _____

Then run §5.4 with everyone else.

5.4 — The Rx Consult Decision Tree

> Two Paths to Act — OTC vs Rx.

Use your domain tally — from §5.3 if you ran the men's track, from §5.3b if you ran the women's track — to find your path. Everyone answers this page.

If 3 or 4 domains flagged "Yes" → Schedule a **full hormone consult**, booked through a free call with the care coordinator at my4mlife.com/consult.

- *Men:* total + free T, SHBG, estradiol, DHEA-S, full cardio markers (lipid, fasting insulin, HbA1c, hs-CRP, homocysteine), GLP-1 candidacy review. This is the TRT + cardio + metabolic consult lane.
- *Women:* estradiol, progesterone, FSH, total + free T, SHBG, TSH, AM cortisol, DHEA-S, paired with the same cardio and metabolic markers and a real sleep evaluation. This is the menopause / hormone-therapy consult lane. Start with a free care coordinator call at my4mlife.com/consult; the coordinator takes your history and labs and lines up the next step. Nobody should leave with nothing.

My decision: □ Scheduling this week. Date booked: _____

If 1 or 2 domains flagged "Yes" → Schedule a **targeted consult** for the most-flagged domain. - Hormones-only? Full panel plus a hormone consult — the TRT track

for men, the menopause / hormone-therapy track for women. - Cardiovascular-only? Metabolic + cardio consult, possible GLP-1. - Cognitive-only? Cognitive workup + prescription gut-barrier protocol evaluation. - Quality-of-life-only? Still book it. This is the domain most often filed under “just stress” for years.

My most-flagged domain: _____

My decision: □ Scheduling this week. Date booked: _____

If all domains are “Mild” or “No” → Continue the **OTC foundation stack** — gut-barrier probiotic, D3+K2 stack, creatine, omega-3+CoQ10 softgel, sleep-support formula, magnesium L-threonate, and the pre-workout creatine+citrulline+beetroot blend on training days. Hold the line on protein-first (30–40g, post-training to open the eating window), the eating window, lifts, sleep, sun. Reassess in 90 days using this same worksheet. See my4mlife.com for the specific products we currently recommend.

My decision: □ Staying OTC. Reassessment date (90 days out): _____

If you or your physician have already diagnosed MCI, early dementia, Alzheimer’s, or Parkinson’s → Engage the **regenerative therapies** consult immediately. This arm is time-sensitive. Every month of delay is tissue you do not get back. See my4mlife.com for the current regenerative protocol.

My decision: □ Booking regenerative therapies consult this week. Date: _____

5.5 — Daily Check-In Tracker

Keep every Week 1 and Week 2 behavior in place. Check the box at the end of each day.

Monday

Behavior	Done
Morning sun + walk, 8-15 min, no sunglasses	☐
Gut-barrier probiotic — with first meal	☐
30–40g protein-first at first meal (after training if trained fasted)	☐
Eating window 9 AM – 6 PM	☐
D3 + K2 stack + creatine with breakfast (creatine around the lift on training days)	☐
Omega-3 + CoQ10 softgel with largest fat meal	☐
Resistance training (if scheduled today)	☐
Magnesium L-threonate 1.5-2g evening	☐
Sleep-support formula 30-60 min before lights out	☐
Lights out by 10:30 PM	☐

Audit work today (5 min on §5.2/§5.3 or §5.2b/§5.3b): ☐

Tuesday

Behavior	Done
Morning sun + walk	☐
Gut-barrier probiotic (first meal)	☐
30–40g protein-first	☐
Eating window held	☐
D3 + K2 stack + creatine	☐
Omega-3 + CoQ10 softgel	☐
Lift / move	☐
Mag L-threonate PM	☐
Sleep-support formula	☐
Lights out 10:30	☐

Audit work today: ☐

Wednesday

Behavior	Done
Morning sun + walk	☐
Gut-barrier probiotic (first meal)	☐
30–40g protein-first	☐
Eating window held	☐
D3 + K2 stack + creatine	☐
Omega-3 + CoQ10 softgel	☐
Lift / move	☐
Mag L-threonate PM	☐
Sleep-support formula	☐
Lights out 10:30	☐
Cohort Zoom attended	☐

Thursday

Behavior	Done
Morning sun + walk	☐
Gut-barrier probiotic (first meal)	☐
30–40g protein-first	☐
Eating window held	☐
D3 + K2 stack + creatine	☐
Omega-3 + CoQ10 softgel	☐
Lift / move	☐
Mag L-threonate PM	☐
Sleep-support formula	☐
Lights out 10:30	☐

Consult booked or OTC reassessment date set: ☐

Friday

Behavior	Done
Morning sun + walk	☐
Gut-barrier probiotic (first meal)	☐
30–40g protein-first	☐
Eating window held	☐
D3 + K2 stack + creatine	☐
Omega-3 + CoQ10 softgel	☐
Lift / move	☐
Mag L-threonate PM	☐
Sleep-support formula	☐
Lights out 10:30	☐

Saturday

Behavior	Done
Morning sun + walk	☐
Gut-barrier probiotic (first meal)	☐
30–40g protein-first	☐
Eating window held	☐
D3 + K2 stack + creatine	☐
Omega-3 + CoQ10 softgel	☐
Lift / move	☐
Mag L-threonate PM	☐
Sleep-support formula	☐
Lights out 10:30	☐

Sunday

Behavior	Done
Morning sun + walk	☐
Gut-barrier probiotic (first meal)	☐
30–40g protein-first	☐
Eating window held	☐
D3 + K2 stack + creatine	☐
Omega-3 + CoQ10 softgel	☐
Lift / move	☐
Mag L-threonate PM	☐
Sleep-support formula	☐
Lights out 10:30	☐
End-of-Week 4M Reflection completed (§5.7)	☐
Adherence Scorecard filled in (§5.8)	☐

5.6 — Weekly Zoom Prep Page

Bring this page to the weekly cohort Zoom. Write the answers before you log on. You do not have to read them aloud — you have to write them.

---

1. Which domain surprised me the most when I rated it honestly?

2. Which domain am I most reluctant to bring up — even in a room of people doing the same work?

3. What is my decision on the Rx consult path?

□ Full hormone consult — booking this week □ Targeted consult — domain: ___ □ **Staying OTC — 90-day reassessment date: _____** □ Regenerative therapies consult — booking this week

The accountability target I named in Week 1 — who I am doing this for — would say this about my decision:

5.7 — End-of-Week 4M Reflection

Hormones live mostly under **Mitigate** (the insults driving the decline) and **Muscle** (the substrate you are trying to rebuild). Weight those two heavier this week.

Mind

The point of every other M is to protect this one. What did this week's audit tell you about your Mind — directly or indirectly?

Cognitive markers from §5.2 #10 / §5.2b #3 and §5.3 / §5.3b Domain 3:

The honest sentence I do not want to write here:

> Mind is the destination.

Muscle

Testosterone, estradiol, growth hormone, IGF-1 — these are the substrate of every muscle fiber, every recovery cycle, every rep that actually adds tissue. Your audit ratings on recovery, training response, and visceral fat are direct readouts on this substrate.

My recovery rating (§5.2 #4), or my 3 PM energy (§5.2b #5): _____ Five years ago: _____

My training response rating (§5.2 #7 / §5.2b #7): _____

My waist trend (§5.2 #8 / §5.2b #6): _____

If these numbers are not where I want them, the upstream lever this month is:

□ Lift harder and more often (compound lifts, 2-3x/week) □ Get the protein in (30–40g first, every feeding window opener) □ Sleep — fragmented sleep degrades every hormone in the panel. In men, most testosterone is made between 11 PM and 3 AM; in women, the 2–3 AM wake-up is often the transition itself. □ Engage the Rx consult — full hormone panel (TRT track for men, menopause / hormone-therapy track for women)

The one I commit to this week:

> Strong body, sharp mind. Resistance training is neuroprotection.

Mitigate

This is the week's center of gravity. The insulting behaviors driving hormonal and vascular decline are mostly choices, not fate.

Insult inventory — check every one that's true this week:

□ Alcohol on more than 2 nights □ Sleep under 7 hours on more than 2 nights □ Visceral fat creeping (waist up vs 5 years ago) □ Untreated snoring / suspected apnea □ Refined seed oils still in the kitchen □ Late eating past 6 PM more than twice this week □ Skipped lifts this week □ Chronic stress with no decompression valve □ Avoided the medical workup I know I need

The ONE I am eliminating first this month (pick one — pick the one that scares you most to write down):

> Stop hurting yourself first. Then add what works.

Motivate

Most people do not book the hormone consult because of pride, not because of money or time.

The face I named as my accountability target in Week 1:

What I owe that person — in plain language — by making the decision in §5.4:

The sentence I would say to them if I were brave this week:

5.8 — Week 3 Adherence Scorecard

Tear-out page — tape this to your bathroom mirror or screenshot for your phone lock-screen.

Behavior	M	T	W	T	F	S	S
Morning sun + walk	☐	☐	☐	☐	☐	☐	☐
Gut-barrier probiotic (first meal)	☐	☐	☐	☐	☐	☐	☐
30–40g protein-first	☐	☐	☐	☐	☐	☐	☐
Eating window 9-6	☐	☐	☐	☐	☐	☐	☐
D3 + K2 stack + creatine	☐	☐	☐	☐	☐	☐	☐
Omega-3 + CoQ10 softgel	☐	☐	☐	☐	☐	☐	☐
Resistance training (2-3x)	☐	☐	☐	☐	☐	☐	☐
Mag L-threonate PM	☐	☐	☐	☐	☐	☐	☐
Sleep-support formula	☐	☐	☐	☐	☐	☐	☐
Lights out 10:30 PM	☐	☐	☐	☐	☐	☐	☐
Cohort Zoom attended			☐				

Week 3 once-only items:

Item	Y / N
Hormone self-audit completed (§5.2 men / §5.2b women)	☐ Y ☐ N
4-Domain Canary Worksheet completed (§5.3 men / §5.3b women)	☐ Y ☐ N
Rx Consult Decision made and recorded (§5.4)	☐ Y ☐ N
Consult booked OR 90-day reassessment date set	☐ Y ☐ N

Days hit 8+ behaviors: _____ / 7

Protocol Score this week: _____ (the ring on your app home screen; higher is better)

My one-line note to next week's me:

> Begin with the end in mind.
> Don't lose your identity and your dignity while you still have a choice.

Part 6 — Week 4: Mitigate, Synthesize, Recommit

6.1 — How Week 4 works

The first three weeks were about building. You added a morning, you added protein, you added lifts, you added sleep structure, you added a face on the fridge. Most of it stuck. Some of it didn't. That's the data.

Week 4 is different. Week 4 strips.

The Mitigate pillar carries one job: *Stop hurting yourself first. Then add what works.* We've been adding for three weeks. Now we audit honestly what you are still doing that is degrading the system underneath everything you've added. The four drinks on Thursday that you called "social." The bedroom router blinking three feet from your head. The 6 AM scroll under the bathroom fluorescents. The nightly edible you told yourself helps you sleep. The third antibiotic course in two years that nobody followed with gut recovery.

These are the insults. Most of them are invisible to you because they're normal in your peer group. They're not normal for a brain you want at 85.

This week does two things.

One: the full Eliminate-the-Insulting-Behavior audit. You will walk every Mitigate category — gut, sleep, environment, hormones, substance use, cognitive — and mark honestly what you're still doing. Then you pick the three insults you're cutting in the next 30 days. Three. Not all of them. Three.

Two: the month-end synthesis. What changed from your Part 1 baseline. What worked and earned its place. What slipped and why. The 90-day plan forward. The locked recommit procedure for when you fall off — because you will, and the people who win are the ones who restart fastest.

> Stop hurting yourself first. Then add what works.
>
> The person who runs the loop longest wins.

Print this section. Sit with it on Sunday morning with coffee and a pen. Don't speed through it. This is the deepest week of Month 1.

6.2 — The Full Eliminate-the-Insulting-Behavior Audit

For each insult: ☐ **Yes** / ☐ **Some** / ☐ **No.** Be honest. Nobody sees this but you and the person you're becoming.

Gut

- ☐ Yes ☐ Some ☐ No — Processed foods and seed oils are part of my daily baseline (chips, fried restaurant food, packaged snacks, "vegetable" oils at home).
- ☐ Yes ☐ Some ☐ No — I've had multiple antibiotic courses in the last five years without a targeted gut recovery protocol after.
- ☐ Yes ☐ Some ☐ No — I average more than 7 alcoholic drinks per week.
- ☐ Yes ☐ Some ☐ No — I have ongoing bloating, food sensitivities, or post-meal brain fog that I've been ignoring or normalizing.

Top gut insult I'm cutting: _____

Sleep

- □ Yes □ Some □ No — I drink alcohol within 3 hours of bed.
- □ Yes □ Some □ No — I'm on screens past 9 PM (phone, TV, laptop) most nights.
- □ Yes □ Some □ No — I eat after 7 PM at least 3 nights a week.
- □ Yes □ Some □ No — I run a sleep debt Monday-Friday and try to repay it on the weekend.

Top sleep insult I'm cutting: _____

Environment

- □ Yes □ Some □ No — I get no direct morning sunlight before screens.
- □ Yes □ Some □ No — I spend my workday under fluorescent lights and in front of screens with no breaks outside.
- □ Yes □ Some □ No — I drink unfiltered tap water (or only filter at the fridge, not at the tap).
- □ Yes □ Some □ No — I have no air filtration in my bedroom or primary workspace.
- □ Yes □ Some □ No — My router, phone, and electronics are powered on within 6 feet of my head at night.

Top environmental insult I'm cutting: _____

Hormones

- □ Yes □ Some □ No — I average less than 7 hours of sleep on weeknights.
- □ Yes □ Some □ No — I drink alcohol daily or near-daily.
- □ Yes □ Some □ No — My stress is chronic and unaddressed (not just busy — *chronic*).
- □ Yes □ Some □ No — I sit more than 8 hours a day and do not train resistance 2-3x/week.
- □ Yes □ Some □ No — I carry visceral fat around my midsection that I haven't seriously addressed.

Top hormonal insult I'm cutting: _____

Substance Use

- □ Yes □ Some □ No — Alcohol more than 7 drinks/week.
- □ Yes □ Some □ No — Nightly cannabis (any route — flower, edible, vape).
- □ Yes □ Some □ No — Prescription sleep aids (Ambien, trazodone, benzos) long-term.
- □ Yes □ Some □ No — Smoking or vaping nicotine.

Top substance insult I'm cutting: _____

Cognitive

- □ Yes □ Some □ No — Chronic stress I have not put a structural tool against (no walk, no breath protocol, no boundary).
- □ Yes □ Some □ No — Zero novel cognitive demand in my week (no new skill, language, instrument, book, course).
- □ Yes □ Some □ No — No resistance training (yes — this is a cognitive insult, not just a muscle one).

Top cognitive insult I'm cutting: _____

The 3 insults I am eliminating in the next 30 days

Not the six. Not the ten. Three. The ones you will actually win.

> ***1.*** _____
>
> ***2.*** _____
>
> ***3.*** _____
>
> Eliminate the insulting behavior.

6.3 — Daily Check-In Tracker

One page per day. Same format as prior weeks. Print and fill by hand.

Monday

Daily Floor	Done
Morning sun + walk (8-15 min, no sunglasses)	☐
30–40g protein-first at first meal	☐
9 AM – 6 PM eating window held	☐
Gut-barrier probiotic (with first meal)	☐
D3 + K2 stack + creatine + omega-3 + CoQ10 softgel (with meal)	☐
Magnesium L-threonate (evening)	☐
Sleep-support formula + lights out by 10:30 PM	☐
Resistance training OR Zone 2 walk	☐
One insult avoided today: _____	☐

One line — what hurt the system today? _____

One line — what helped? _____

Tuesday

Daily Floor	Done
Morning sun + walk	☐
30–40g protein-first	☐
9 AM – 6 PM window	☐
Gut-barrier probiotic	☐
D3 + K2 stack + creatine + omega-3 + CoQ10 softgel	☐
Magnesium L-threonate	☐
Sleep-support formula + 10:30 PM lights out	☐
Training OR Zone 2	☐
Insult avoided: _____	☐

Hurt: _____ Helped: _____

Wednesday

Daily Floor	Done
Morning sun + walk	☐
30–40g protein-first	☐
9 AM – 6 PM window	☐
Gut-barrier probiotic	☐
D3 + K2 stack + creatine + omega-3 + CoQ10 softgel	☐
Magnesium L-threonate	☐
Sleep-support formula + 10:30 PM lights out	☐
Training OR Zone 2	☐
Insult avoided: _____	☐
Cohort Zoom attended	☐

Hurt: _____ Helped: _____

Thursday

Daily Floor	Done
Morning sun + walk	☐
30–40g protein-first	☐
9 AM – 6 PM window	☐
Gut-barrier probiotic	☐
D3 + K2 stack + creatine + omega-3 + CoQ10 softgel	☐
Magnesium L-threonate	☐
Sleep-support formula + 10:30 PM lights out	☐
Training OR Zone 2	☐
Insult avoided: _____	☐

Hurt: _____ Helped: _____

Friday

Daily Floor	Done
Morning sun + walk	☐
30–40g protein-first	☐
9 AM – 6 PM window	☐
Gut-barrier probiotic	☐
D3 + K2 stack + creatine + omega-3 + CoQ10 softgel	☐
Magnesium L-threonate	☐
Sleep-support formula + 10:30 PM lights out	☐
Training OR Zone 2	☐
Insult avoided: _____	☐

Hurt: _____ Helped: _____

Saturday

Daily Floor	Done
Morning sun + walk	☐
30–40g protein-first	☐
9 AM – 6 PM window	☐
Gut-barrier probiotic	☐
D3 + K2 stack + creatine + omega-3 + CoQ10 softgel	☐
Magnesium L-threonate	☐
Sleep-support formula + 10:30 PM lights out	☐
Training OR Zone 2	☐
Insult avoided: _____	☐

Hurt: _____ Helped: _____

Sunday

Daily Floor	Done
Morning sun + walk	☐
30–40g protein-first	☐
9 AM – 6 PM window	☐
Gut-barrier probiotic	☐
D3 + K2 stack + creatine + omega-3 + CoQ10 softgel	☐
Magnesium L-threonate	☐
Sleep-support formula + 10:30 PM lights out	☐
Training OR Zone 2	☐
Insult avoided: _____	☐
Sunday synthesis review (§6.5)	☐

Hurt: _____ **Helped:** _____

6.4 — Weekly Zoom Prep

Bring honest answers. Not polished ones.

1. One thing I now understand about myself that I did not understand four weeks ago:

2. One product or protocol that meaningfully changed something this month (what changed, and how I know):

3. One protocol I am dropping because it did not earn its place:

4. The single biggest insult I am cutting starting this week:

6.5 — Month-End Synthesis

The deepest reflection in the logbook. Block 60 minutes. Pen, coffee, no phone.

Page 1 — What changed

Pull your Part 1 baseline. Rate each again today on the same 0-5 scale. Write the delta.

Audit categories (0 = no issue, 5 = daily and affecting my life. Same scale as Part 1; a lower Day 30 is the win.)

Category	Day 1	Day 30	Delta
Cognitive / brain fog	___	___	___
Sleep	___	___	___
Mood	___	___	___
Gut health	___	___	___
Movement & strength	___	___	___
Weight	___	___	___
Alcohol	___	___	___
Hormones	___	___	___
Sexual function	___	___	___
Purpose & accountability	___	___	___

Your canary, re-rated. Go back to §5.2 or §5.2b. Copy your three lowest markers and rate them again today.

Marker	Week 3	Day 30	Delta
______	___	___	___
______	___	___	___
______	___	___	___

Physiology baselines

Metric	Day 1	Day 30	Delta
Weight	___	___	___
Waist (at navel)	___	___	___
Resting HR	___	___	___
Blood pressure	___	___	___
Avg sleep hours	___	___	___
Avg drinks/week	___	___	___

The single biggest shift I felt this month:

Page 2 — What worked

Three things from this month that earned a permanent spot in the long-term protocol. Be specific. Not "supplements." *Which one, and what did it do for you.*

1. _____

Why it earned its place: _____

2. _____

Why it earned its place: _____

3. _____

Why it earned its place: _____

Page 3 — What slipped, and why

Three things you tried that didn't stick. The *why* matters more than the *what*. Was it knowledge (you didn't really understand the mechanism)? Willpower (you understood and didn't do it)? Structure (no calendar slot, no trigger, no cue)? Or wrong product / wrong protocol for you?

1. What slipped: _____

Why (knowledge / willpower / structure / wrong fit): _____

What I'll change: _____

2. What slipped: _____

Why: _____

What I'll change: _____

3. What slipped: _____

Why: _____

What I'll change: _____

Page 4 — The 90-day plan

Continue (the behaviors and products carrying me):

Add (the next layer — pick from Mitigate insults, Muscle progression, Mind training, Motivate structure):

Drop (didn't earn its place):

Three sentences — what I am optimizing for over the next 90 days:

6.6 — The 30-Day Recommit Procedure

You will fall off. Everyone on this protocol falls off. The wedding, the work trip, the flu, the funeral, the stretch where life punches you in the mouth and the daily floor goes to zero for nine days.

This is the locked procedure. Tape it inside the logbook cover.

1. **Do not try to make it up.** No double-stacking after a slip. You cannot earn back lost days by punishing the next ones. Trying to "catch up" is the second insult after the first.
2. **Pick ONE behavior.** Whichever has the lowest activation energy for *you* right now. For most people it's the morning sun walk. For some it's protein-first. For some it's lights out at 10:30. Pick the easiest re-entry, not the most important one.

3. **Run that one for 3 days.** Just that one. Don't add. Don't apologize. Don't track anything else. Three clean days of the easiest behavior.
4. **Add the second one on day 4.** Now two behaviors. Run both for three days.
5. **Stack until full compliance by day 13-14.** By the end of the second week you are back on the full daily floor. The people who get back here in two weeks beat the ones who tried to come back at 100% on day one and broke again by day three.
6. **Show up to the weekly Zoom even when you have run zero behaviors that week.** Especially then. The face on screen matters more than the report card. The cohort sees you. You see the cohort. The loop stays closed.

> The person who runs the loop longest wins.

Recommit pre-commitment

Fill this in *now*, while you're on the protocol, not later when you're off it.

> ***The next time I fall off, the ONE behavior I will restart with is:***
>
> ***The reason it has the lowest activation energy for me is:***
>
> ***The person I will text on the day I restart (accountability target):***

6.7 — Final 4M Reflection

One page per pillar, in locked order. Longer than the weekly versions. Sit with each one.

Mind — what we're protecting

Mind is the destination. Mind is also where the cycle starts. Four weeks ago you named a face. The person you want to be at 85 is sharp, present, knowing the names of grandchildren you may not have met yet.

What does my Mind feel like today versus 30 days ago?

The cognitive behaviors I am keeping for the next 90 days (training, learning, stress structure, sleep architecture):

The end I am beginning with:

Muscle — the engine

> Strong body, sharp mind. Resistance training is neuroprotection.
>
> ***My training consistency this month (sessions completed / sessions planned):*** _____ / _____
>
> ***The lift, food, or stack that produced the biggest shift in how I feel:***
>
> ---
>
> ***My 90-day strength goal (specific — a lift number, a body comp marker, a stair-climb test):***
>
> ---

Mitigate — stop hurting yourself first

> ***The three insults I named in §6.2 — restated here so I see them every page-flip:***
>
> 1. _____
> 2. _____
> 3. _____
>
> ***The insult that scares me most to cut (and why):***
>
> _____
>
> _____

Motivate — what sustains the work

> ***The face I am doing this for (re-name them, even if they haven't changed):***
>
> ---
>
> ***The structural enabler I am adding in Month 2 (calendar block, gym partner, automated re-order, kitchen overhaul, alcohol-free home, etc.):***
>
> ---
>
> ***Why this matters beyond me:***
>
> ---
>
> ---

6.8 — Week 4 Adherence Scorecard + Monthly Summary

Page 1 — Week 4 Adherence Scorecard

Tear-out page — tape this to your bathroom mirror or screenshot for your phone lock-screen.

Daily Floor Behavior	M	T	W	T	F	S	S	Total
Morning sun + walk	☐	☐	☐	☐	☐	☐	☐	__ /7
30–40g protein-first	☐	☐	☐	☐	☐	☐	☐	__ /7
9 AM – 6 PM window	☐	☐	☐	☐	☐	☐	☐	__ /7
Gut-barrier probiotic (first meal)	☐	☐	☐	☐	☐	☐	☐	__ /7
D3 + K2 stack + creatine + omega-3 + CoQ10 softgel	☐	☐	☐	☐	☐	☐	☐	__ /7
Magnesium L-threonate PM	☐	☐	☐	☐	☐	☐	☐	__ /7
Sleep-support formula + 10:30 lights out	☐	☐	☐	☐	☐	☐	☐	__ /7
Resistance OR Zone 2	☐	☐	☐	☐	☐	☐	☐	__ /7
One insult avoided	☐	☐	☐	☐	☐	☐	☐	__ /7

Weekly Zoom attended: ☐

Week 4 score: _____ / 63

Protocol Score this week: _____ (the ring on your app home screen; higher is better)

Page 2 — Month 1 Summary Scorecard

Tear-out page — keep this in the front of your logbook.

Daily Floor Behavior	Wk 1	Wk 2	Wk 3	Wk 4	Month Total
Morning sun + walk	__ /7	__ /7	__ /7	__ /7	__ /28
30–40g protein-first	__ /7	__ /7	__ /7	__ /7	__ /28
9 AM – 6 PM window	__ /7	__ /7	__ /7	__ /7	__ /28
Gut-barrier probiotic (first meal)	__ /7	__ /7	__ /7	__ /7	__ /28
D3 + K2 stack + creatine + omega-3 + CoQ10 softgel	__ /7	__ /7	__ /7	__ /7	__ /28
Magnesium L-threonate PM	__ /7	__ /7	__ /7	__ /7	__ /28
Sleep-support formula + 10:30 lights out	__ /7	__ /7	__ /7	__ /7	__ /28
Resistance OR Zone 2	__ /7	__ /7	__ /7	__ /7	__ /28
Insult avoided	__ /7	__ /7	__ /7	__ /7	__ /28
Weekly Zoom attended (1 pt)	__ /1	__ /1	__ /1	__ /1	__ /4

Month 1 total score: _____ / 256

A score above 200 means you ran the loop. A score below 150 means the structure isn't holding yet — Month 2 starts with the recommit procedure in §6.6, not with new

behaviors.

> Begin with the end in mind.
> Don’t lose your identity and your dignity while you still have a choice.
> Stop hurting yourself first. Then add what works.
> Eliminate the insulting behavior.
> The person who runs the loop longest wins.

— Dr. TJ

Part 7 — The Stack Reference

This is the part you come back to. When you forget why a powder is sitting on your counter, when your spouse asks what the capsule does, when you're deciding whether to add the Rx tier — open here.

How to read this section

Two ideas govern everything that follows.

No weak links. Every product in the My4MLife stack — ours and the affiliates we point you at — passes a full-ingredient audit. Not a marketing audit. An ingredient-by-ingredient audit. One off-message ingredient disqualifies the entire SKU. A "premium" protein bar with soybean oil is out. A "clinical" multivitamin with titanium dioxide is out. A "clean" sleep formula with proprietary blends hiding under-dosed actives is out. The reason is simple: you cannot eliminate the insulting behavior with one hand while feeding it with the other. The stack you're about to read has already been filtered. You don't have to do that work again.

Two Paths to Act. Almost every job in the stack has an OTC path and an Rx path. OTC is the daily foundation — what every Protégé runs. Rx is the single-active compounded add-on, prescribed through a licensed telemed partner when the indication is there. You don't start on the Rx tier. You earn it by running the foundation, surfacing what the

foundation can't reach, and bringing labs or symptoms that justify the upgrade. Both paths are real. Neither is required to begin.

What follows is one page per product, in the order you'd actually add them. Then a one-page pairing matrix.

Gut-Barrier Probiotic (the gut-brain seal)

Tier: OTC daily foundation **When:** With or immediately after your first meal of the day **Dose:** As labeled **What it is:** The gut-brain seal, your daily over-the-counter gut-barrier step. See my4mlife.com/stack for the specific product we currently recommend. **Why it's in the stack:** This is the anchor. The gut lining is where neuroinflammation starts for most people in midlife — leaky barrier, endotoxin translocation, vagal-nerve signaling junk that ends up looking like brain fog, mood drift, and sleep fragmentation. This formula seals the barrier, calms the local inflammation, and quiets the upstream input to the brain. Nothing else in the stack works as well on a leaky gut. **What it pairs with:** Everything. Specifically amplifies the D3+K2 stack (fat-soluble absorption), the omega-3+CoQ10 softgel (anti-inflammatory synergy), and any nootropic peptide (cleaner substrate to act on). **What to watch for in the first 30 days:** Bowel regularity normalizes in 7-14 days. Bloating and post-meal fog drop noticeably by week 3. **Two Paths note:** Pairs with our proprietary compounded gut-barrier prescription (physician-written to your needs) on top — same anchor, deeper repair. See my4mlife.com.

> The gut-brain seal that protects cognitive longevity.

Vitamin D + K2 + Boron + Astaxanthin Stack

Tier: OTC daily foundation **When:** With protein-first breakfast **Dose:** 1 capsule daily **What it is:** D3 (cholecalciferol) + K2 (MK-7) + boron + astaxanthin. See my4mlife.com for the specific product we currently recommend. **Why it's in the stack:** Four fat-soluble defenders that nearly everyone in midlife is under-dosed on. D3 runs immune, mood, and hormonal signaling. K2 routes calcium to bone instead of artery wall — the missing partner that makes D3 safe at clinical doses. Boron supports free testosterone and bone matrix. Astaxanthin is the most powerful lipid-phase antioxidant we have access to, and it crosses the blood-brain barrier. Together they form a daily structural defense layer. **What it pairs with:** Omega-3 + CoQ10 softgel (shared fat-soluble absorption window, mitochondrial synergy). Gut-barrier probiotic (sealed gut means D3/K2 actually get absorbed). **What to watch for in the first 30 days:** Subtle but real — mood floor lifts, recovery from training improves, skin tone shifts. If you're starting from a deficient D level, expect the biggest change at the 6-8 week mark when serum levels normalize. **Two Paths note:** No Rx upgrade. This is foundation only.

Creatine Monohydrate

Tier: OTC daily foundation **When:** Around the lift on training days, with breakfast otherwise. Year-round, no cycling. **Dose:** 5g daily (no loading phase needed) **What it is:** Pure creatine monohydrate. Not HCL, not buffered, not "advanced." Monohydrate. The form that has 700+ trials behind it. **Why it's in the stack:** This is the single most-evidenced cognitive supplement in midlife. It's not a muscle supplement that happens to help the brain — it's a brain supplement that also happens to build muscle. Creatine refuels ATP in neurons under metabolic stress, which is exactly what an aging brain runs into during cognitive load, sleep loss, and recovery from exercise. **What it pairs with:** Resistance training (non-negotiable). Electrolyte powder (zero-sugar, low-sodium; see my4mlife.com for the one we use) on training days — half a scoop with your 5 g of creatine in 16 oz water before the fasted lift, the other half after; it also blunts appetite if

you want to push the fast further into the day. Non-training days: 5 g with breakfast. Pre-workout creatine+citrulline+beetroot blend on training days (stacked NO + ATP support). Omega-3 + CoQ10 softgel (the cardio-neuro-energetic trio: omega membranes + ubiquinol mitochondria + creatine ATP). **What to watch for in the first 30 days:** 1-2 pounds of water weight in the first 10 days — that's intracellular hydration, not fat. Strength on compound lifts goes up within 2-3 weeks. Mental clarity under sleep deprivation is the tell that it's reached your brain.

> If you take exactly one supplement from this stack, take this one.

Two Paths note: No Rx upgrade. Foundation only.

Omega-3 + Ubiquinol CoQ10 Softgel

Tier: OTC daily foundation **When:** With the largest fat-containing meal of the day **Dose:** 2 softgels daily **What it is:** EPA 1200 mg + DHA 800 mg in triglyceride (TG) form, IFOS 5-star certified for purity and oxidation. Plus ubiquinol 200 mg — the reduced, bioavailable form of CoQ10. (Cardio-neuro axis coverage — the two systems share a membrane.) See my4mlife.com for the specific product we currently recommend. **Why it's in the stack:** Two jobs in one capsule. The omega side rebuilds neuronal membrane fluidity and damps systemic inflammation — DHA is literally a structural lipid in your brain. The ubiquinol side restores mitochondrial electron-transport-chain efficiency, which collapses with age and with any statin exposure. Together they cover the cardio-neuro axis at the membrane + energy level. **What it pairs with:** D3+K2 stack (fat-soluble absorption window). Creatine (full ATP-restoration trio). Any GLP-1 protocol (counters the lipid-quality drop common on aggressive fat-loss phases). **What to watch for in the first 30 days:** Joint stiffness eases by week 2. Blood pressure drifts down a few points in the responsive. If you've been on a statin, expect the biggest energy/cognition lift to come from the ubiquinol side at the 4-6 week mark. **Two Paths**

note: No direct Rx upgrade. The Rx-tier circulation play is the nattokinase prescription (alongside the sleep-support formula).

Sleep-Support Formula

Tier: OTC daily foundation **When:** 30-60 minutes before lights out (lights out by 10:30 PM) **Dose:** 1 serving nightly **What it is:** Sleep-architecture support — non-melatonin-led formula designed to deepen slow-wave and REM without the next-morning hangover or the receptor-downregulation problem of chronic high-dose melatonin. See my4mlife.com for the specific product we currently recommend. **Why it's in the stack:** Sleep is where the glymphatic system clears the brain. No clearance, no cognitive longevity. Most people in midlife are not sleep-deprived in hours — they're sleep-deprived in architecture. This formula exists to fix the architecture so the hours you do sleep actually count. **What it pairs with:** Magnesium L-threonate — taken together they form the sleep stack. Lights-out-by-10:30 protocol. Morning sun anchor (sleep is set in the morning, not the evening). **What to watch for in the first 30 days:** First week is the tell. Fewer 3 AM wake-ups. Dreams come back (a sign REM is recovering). Morning HRV trends up if you wear a tracker. **Two Paths note:** Pairs with a nattokinase prescription for the circulation/clotting tier — different mechanism, same sleep-and-cardiovascular protective intent.

Magnesium L-Threonate

Tier: OTC daily foundation **When:** Evening, 1-2 hours before bed **Dose:** 1.5-2 g (delivers ~144-192 mg elemental magnesium) **What it is:** Magnesium L-threonate. This matters: it is the only magnesium form with demonstrated hippocampal bioavailability. Glycinate, citrate, and oxide do not cross into the brain the way threonate does. **Why it's in the stack:** Hippocampal magnesium drives synaptic density — the physical

infrastructure of memory. Restoring it has been shown to improve working memory and executive function in older adults. This is the magnesium that targets the brain. Use other forms for bowels and muscle if you want; use threonate for the mind. **What it pairs with:** Sleep-support formula (the sleep stack). Lion's mane (synaptic-density synergy). Any nootropic peptide cycle. **What to watch for in the first 30 days:** Calmer evenings. Sharper word-finding. The "where did I put my keys" frequency drops. Real effect builds over 8-12 weeks — this is a remodeling supplement, not a stimulant. **Two Paths note:** No Rx upgrade. Foundation only.

Pre-Workout Creatine + Citrulline + Beetroot + Electrolyte Blend

Tier: OTC situational (pre-workout) **When:** 20-30 minutes before resistance training or a hard walk **Dose:** 1 scoop in 12-16 oz water **What it is:** Creatine + L-citrulline + beetroot + electrolytes. Monk-fruit sweetened. Fast-compatible (no insulinogenic load). See my4mlife.com for the specific product we currently recommend. **Why it's in the stack:** Mitochondrial ATP plus endothelial nitric oxide in one scoop. Citrulline and beetroot raise NO and blood flow to working muscle and brain. The added creatine layer stacks on top of your daily 5g for training days. Electrolytes prevent the cramps and dizziness that derail older lifters mid-set. **What it pairs with:** Daily creatine (additive on training days). Resistance training (the entire point). Omega-3 + CoQ10 softgel (endothelial health amplifies citrulline's effect). **What to watch for in the first 30 days:** Better pumps and longer training tolerance in week 1. Lower perceived exertion on cardio. Pre-workout brain clarity — citrulline + beetroot reach the brain too. **Until the blend is in your hands:** a plain zero-sugar, low-sodium electrolyte powder covers the electrolyte part of this on its own — half a scoop with your creatine before the fasted lift, the other half after. **Two Paths note:** No Rx upgrade. Situational foundation.

Lion’s Mane

Tier: OTC situational (cognitive add) **When:** Morning with breakfast **Dose:** 1000 mg/day of a verified fruiting-body extract (not mycelium-on-grain) **What it is:** Hericium erinaceus fruiting-body extract, standardized to hericenones and erinacines. **Why it’s in the stack:** NGF and BDNF support — the growth factors that maintain neuronal arborization and synaptic plasticity in the aging brain. Best used cycle-based: 8 weeks on, 2 weeks off. Not a daily-forever requirement; a layered tool you bring in when you want a cognitive push. **What it pairs with:** Magnesium L-threonate (synaptic density). Cerebrolysin cycles (BDNF stacking). Gut-barrier probiotic (clean gut means cleaner mushroom-extract absorption — and lion’s mane is gut-active too). **What to watch for in the first 30 days:** Word fluency improves around week 3. If you don’t notice anything at 6 weeks, the SKU is probably mycelium-on-grain — switch brands. **Two Paths note:** No Rx upgrade. Cycle-based situational.

Prescription Gut-Barrier Protocol (proprietary compounded Rx)

Tier: Rx single-active add-on (compounded via licensed telemed partner) **When:** Per prescription, on top of the OTC gut-barrier probiotic **Dose:** Per prescription **What it is:** A proprietary compounded gut-barrier prescription, written by a physician to your individual needs. Pairs on top of the OTC anchor. See my4mlife.com for the current Rx gut protocol. **Why it’s in the stack:** When the OTC seal is doing its job but you have a longer history of insult — NSAID years, antibiotic courses, alcohol decades, prior ulcer, chronic reflux — the prescription adds a deeper repair signal that the OTC tier can’t reach. This is the deeper-tier gut repair lane. **What it pairs with:** Gut-barrier probiotic (the anchor pair — never run Rx without the OTC base). D3+K2 stack (D3 amplifies barrier repair). **What to watch for in the first 30 days:** Reflux and post-meal discomfort drop sharply. Stool quality improves further. Energy and mood floor lifts as endotoxin load drops. **Two Paths note:** This *is* the Rx path for the gut anchor.

Nattokinase Prescription (Sleep + Circulation Rx Tier)

Tier: Rx single-active add-on **When:** Evening, alongside the sleep-support formula **Dose:** Per prescription **What it is:** Nattokinase, compounded. See my4mlife.com for the current sleep Rx protocol. **Why it's in the stack:** Fibrinolytic support — addresses the circulation/clotting tier that becomes a real concern in people with vascular risk factors, post-COVID profiles, or family history of stroke. Sleep and clean cerebral circulation are both required for glymphatic clearance. **What it pairs with:** Sleep-support formula (architecture). Omega-3 + CoQ10 softgel (vascular health). D3+K2 stack (K2 calcification management). **What to watch for in the first 30 days:** Subtler than the gut Rx. Track HRV and morning resting heart rate. Look for steadier readings, not a single dramatic shift. **Two Paths note:** This is the Rx path for circulation. Not a sedative.

TRT (Testosterone Replacement)

Tier: Rx single-active, lab-gated **When:** Per protocol (typically twice-weekly injection or daily cream) **Dose:** Titrated to free T, total T, estradiol, and hematocrit **What it is:** Bio-identical testosterone, prescribed and monitored through the consult pathway. Always paired with full lab work — including estradiol and CBC — and coach-led for the lifestyle work that determines whether TRT actually does its job. **Why it's in the stack:** Low testosterone is the silent multiplier under most of what we treat: visceral fat, sarcopenia, mood drift, brain fog, ED as canary. Restoring physiological T is not optional for many men over 50 — it's foundational. But it is never a shortcut. It works because the lifting, protein, and sleep work. **What it pairs with:** Resistance training (non-negotiable — without lifting, TRT becomes a liability). D3+K2+boron stack (boron + D3 free-T support). Omega-3 + CoQ10 softgel (cardiovascular protection). Gut-barrier probiotic (clean gut, cleaner hormone metabolism). **What to watch for in the first 30 days:**

Energy and libido often shift in week 2-3. Body composition takes 8-12 weeks. Mood floor lifts. Watch hematocrit at 90 days. **Two Paths note:** The OTC adjacent layer is the D3+K2+boron stack + lifting + protein-first eating. Many men optimize free T enough on that alone to not need TRT. The lab tells.

Menopause Hormone Therapy (Rx)

Tier: Rx, lab- and history-gated **When:** Per protocol, daily (route and schedule set by the prescriber) **Dose:** Individualized — titrated to symptoms and labs, reviewed on a set cadence **What it is:** Bio-identical estradiol and progesterone, dosed and routed to the individual, prescribed and monitored through the consult pathway; low-dose testosterone where indicated; and non-hormonal prescription options for women who cannot or do not want to take hormones — approaches for hot flashes, targeted options for genitourinary symptoms, and behavioral treatment for menopausal insomnia. Booked through the care coordinator at my4mlife.com. **Why it's in the stack:** Estradiol is a neurological hormone, not just a reproductive one — it supports cerebral blood flow, neuronal glucose use, synaptic plasticity, and microglial calm. When it falls over a compressed handful of years, sleep, cognition, mood, body composition, bone, and endothelial function all move at once. Timing appears to matter: near the transition, with a healthy vascular system, is a different proposition than a decade later. **What it pairs with:** Resistance training (non-negotiable — bone and lean mass run the same equation). D3+K2+boron stack (K2 is arguably *more* important post-menopause: losing estrogen accelerates both bone loss and calcium migration into arterial walls). Creatine (most women over 50 have never taken it and start from lower baseline stores). Omega-3 + CoQ10 softgel. Gut-barrier probiotic. **What to watch for in the first 30 days:** Sleep continuity is usually the first thing to move. Vasomotor symptoms and mood follow. Cognitive clarity is slower and harder to self-rate — use the §5.2b scores as your baseline and re-rate at 90 days. **Two Paths note:** The OTC adjacent layer is the D3+K2+boron stack, creatine, protein-first eating, lifting, and the sleep architecture from Week 2 — real leverage, and the right first move for many women. Hormone therapy is individualized

and it is not for everyone: a personal history of breast or uterine cancer, a clotting history, and certain cardiovascular and liver conditions all bear on it. That call belongs to a licensed clinician reviewing your history — not to this page. The lab and the history tell.

GLP-1 (Semaglutide / Tirzepatide / Retatrutide)

Tier: Rx single-active, BMI + comorbidity-gated **When:** Weekly injection, per protocol **Dose:** Titrated per molecule and tolerance **What it is:** GLP-1 (or GLP-1/GIP, or GLP-1/GIP/glucagon triple agonist) prescribed through the consult pathway with structured titration and stop-criteria. **Why it's in the stack:** Visceral fat is neuroinflammatory. Insulin resistance is upstream of cognitive decline. For the right candidate — BMI threshold met, comorbidities present, lifestyle effort already in motion — GLP-1 collapses years of struggle into months. It is not a vanity drug. It is a metabolic-and-neurological intervention. **What it pairs with:** Resistance training (mandatory — without lifting, you lose muscle, not fat). 30–40g protein-first rule (mandatory — appetite is suppressed; protein priority is how you keep lean mass). Omega-3 + CoQ10 softgel (lipid quality protection). Gut-barrier probiotic (GLP-1 slows gut motility; the seal matters more, not less). **What to watch for in the first 30 days:** Appetite drops fast. Hydration discipline becomes critical. Watch for under-eating protein — the most common failure mode. **Two Paths note:** The OTC adjacent layer is the 9 AM – 6 PM eating window (train fasted, break the fast with 30–40g protein after) plus lifting. Many people get most of the metabolic benefit there.

Nootropic Peptides

Tier: Rx single-active, cycle-based **When:** Per protocol **Dose:** Per protocol **What it is:** The cognitive-tier Rx lane. Four primary tools: - **Cerebrolysin** — neurotrophic peptide complex, IM, cycle-based (typically 10-20 day cycles). The heaviest cognitive lift in the

lineup. - **Semax** — nasal spray. Daily focus, BDNF support, attention. - **Selank** — nasal spray. Calm-focus, anxiolytic without sedation. Pairs well with Semax. - **Ipamorelin** — GH secretagogue. Recovery, sleep depth, body composition. Evening.

Why it's in the stack: When the foundation is running clean and you want the next cognitive lever — or when there's existing MCI, post-concussive history, or a hard need to protect a specific window — peptides are the tier. They are not first-line. They sit on top of a clean foundation. **What it pairs with:** Gut-barrier probiotic (clean substrate). Magnesium L-threonate (synaptic infrastructure). Omega-3 + CoQ10 softgel (membrane substrate). Sleep stack (recovery substrate). **What to watch for in the first 30 days:** Cycle-dependent. Cerebrolysin: cognitive lift across the cycle, often a residual benefit lasting weeks. Semax/Selank: same-day effect, evaluate within 2 weeks. Ipamorelin: sleep depth and recovery within 2-3 weeks. **Two Paths note:** This is the Rx path. The OTC adjacent layer is lion's mane + magnesium L-threonate + creatine.

Regenerative Therapies (the Regenerative Protocol)

Tier: Regenerative **When:** Per protocol — nationwide mobile delivery, scheduled **Dose:** Per the current regenerative protocol **What it is:** Advanced regenerative medicine, delivered with nationwide mobile service — the team comes to you. See my4mlife.com for the current protocol and delivery details. **Why it's in the stack:** For the already-diagnosed — MCI, early dementia, Alzheimer's, Parkinson's — and for those with biomarker or imaging evidence of accelerated neurodegeneration. The regenerative protocol is time-sensitive. The earlier in the disease curve, the more there is to preserve. This is not a maintenance product. It is the regenerative tier and it belongs to the people who need it now. **What it pairs with:** The entire foundation stack — gut-barrier probiotic, D3+K2+boron stack, omega-3+CoQ10 softgel, creatine, magnesium L-threonate, the sleep stack. The cleaner the substrate, the better the regenerative response. Nootropic peptide cycles are a natural follow-on. **What to watch for:** This is not

evaluated in 30-day increments. Quarterly cognitive testing and imaging mark the timeline. **Two Paths note:** Stands alone at the top of the ladder. There is no OTC analogue.

The Pairing Matrix

One page. Tape it inside the cabinet where the bottles live.

Stack pair	Why they amplify
Gut-barrier probiotic ↔ Prescription gut-barrier protocol	Anchor pair. OTC seals the barrier; Rx drives regenerative repair on top. Never run Rx without the OTC base.
D3+K2+boron stack + Omega-3+CoQ10 softgel	Mitochondrial + membrane support. Fat-soluble absorption window shared. D3/K2/astaxanthin + EPA/DHA/ubiquinol = the structural defense layer.
Creatine + Resistance Training + Pre-workout citrulline+beetroot blend	The muscle stack. Daily creatine builds the baseline ATP pool; lifting demands it; the pre-workout blend stacks NO + ATP + electrolytes on training days.
Magnesium L-Threonate + Sleep-support formula	The sleep stack. Threonate drives hippocampal synaptic density; the sleep formula drives architecture. Lights out by 10:30.
Cerebrolysin + Gut-barrier probiotic + D3+K2 stack	The cognitive optimization stack. Clean gut + fat-soluble defense + neurotrophic Rx. Peptides work best on a clean substrate.
Menopause hormone therapy + Lifting + Protein-First	Same foundation, different alarm. Bone and lean mass run the same equation; the first step is a free care coordinator call, and the plan is individualized.
TRT + Lifting + Protein-First	Hormonal foundation. TRT without lifting is a liability; lifting without protein is futile; the three together are the muscle-mind axis.
GLP-1 + Protein-First (30–40g) + Lifting + Omega-3+CoQ10 softgel	Metabolic foundation. Appetite drops; protein and lifting preserve lean mass; omega+ubiquinol protect cardio during fast fat loss.
Sleep-support formula + Nattokinase Rx + Omega-	Cerebrovascular nighttime stack. Architecture + fibrinolysis + membrane health. The glymphatic

Stack pair	Why they amplify
3+CoQ10 softgel	clearance trio.
Regenerative protocol + full foundation	Substrate matters most here. Every foundation product makes the regenerative tier work better.

> Begin with the end in mind. The stack is not a shopping list. It is a system aimed at one outcome — the best mind possible until your last day of life.

Part 8 — The Advanced Layer (Optional)

This section is the menu, not the mandate.

The people who get the most out of this protocol are the ones who run Week 1–4 honestly, lock in the foundation across all four pillars, and then — somewhere around month three or four — start to ask what's possible if they go further. This section is for them.

You do not need any of it to win. The foundation does 80% of the work. The cognitive optimization tier, the peptide layer, the biofeedback, the cold plunge — these are levers you can pull *on top of* a clean foundation. They are not substitutes for it.

> *Only after the foundation is locked. The person who layers this on a broken foundation gets the bill, not the benefit.*

Read this section now if you want. Commit to running the basics for 90 days before deploying anything from it.

8.1 — The Nootropic Peptide Layer

The Rx tier for cognitive optimization. Coach-led, lab-monitored, prescribed through our licensed telemedicine partners. Built on top of the foundation, not in place of it.

Cerebrolysin

Porcine brain-derived neuropeptide cocktail. IM injection cycles, typically 10–20 days, then rest periods. The most-evidenced regenerative neuropeptide in the clinical literature for post-stroke and early cognitive decline (Heiss, Muresanu, Lindenberg). Position: the most aggressive non-stem-cell intervention for cognitive decline in early to moderate stages. Run 2–3 cycles per year.

Semax

Russian ACTH-derivative nasal peptide. Strong on focus, mood, executive function, neuroprotective in ischemia models. The daily "stimulant alternative" for executive-function days.

Selank

The anxiolytic + pro-cognitive counterpart to Semax. Calm-focus without sedation. Nasal, daily as needed.

BPC-157

Gut + soft tissue + neuroinflammation modulation. Oral for gut-brain axis work; IM for soft-tissue recovery. Here it's used for cognitive purpose.

Ipamorelin

GH secretagogue. Indirectly cognitive via deeper sleep architecture and IGF-1 support. Evening dose, sub-Q.

Am I a peptide candidate?

Check honestly:

□ I have run the foundation cleanly for 90+ days

□ My hormone panel is optimized (or in active correction)

□ My sleep is consistently 7+ hours with deep architecture

□ I have a coach or telemed partner who will run labs and titrate

□ I'm comfortable with injection or nasal administration

□ I understand cycle-based vs continuous protocols

If all six are checked, you're a candidate. If fewer than six, return to the foundation.

8.2 — Brain Training

Be honest about evidence: most consumer "brain training" apps don't transfer to real cognitive function. Most brain games make you better at the games and not much else.

Three approaches have actual evidence:

Dual N-back. Free implementations exist (BrainScale and others). 20 minutes per day, 4–6 weeks, shows working-memory and fluid-intelligence improvements in many studies. Cheap, modest but real effect.

A clinically studied brain-training program. Paid. See my4mlife.com for the one we currently recommend.

Novel skill acquisition. Learning a new language, learning an instrument, learning a new sport, learning to dance — the highest-evidence form of "brain training" in the literature. Real cognitive demand under conditions of slow mastery. Run one for the rest of your life.

Pick **one** for the next 90 days. Just one.

> ***My chosen training (next 90 days):*** _____
>
> ***Frequency target:*** _____ *sessions per week,* _____ *minutes per session*

30-day brain training tracker

Day	Done?	Notes
1	☐	
2	☐	
3	☐	
4	☐	
5	☐	
6	☐	
7	☐	
8	☐	
9	☐	
10	☐	
11	☐	
12	☐	
13	☐	
14	☐	
15	☐	
16	☐	
17	☐	

Day	Done?	Notes
18	☐	
19	☐	
20	☐	
21	☐	
22	☐	
23	☐	
24	☐	
25	☐	
26	☐	
27	☐	
28	☐	
29	☐	
30	☐	

8.3 — Biofeedback

HRV training. The single most-evidenced consumer biofeedback intervention. Slow-paced breathing (roughly 6 breaths per minute) trains vagal tone over weeks. Any HRV tool with biofeedback mode works. **Five minutes per day. Daily.** Add it to the morning sun walk or wind-down routine.

EEG meditation aid. Real-time feedback during meditation. Worth running for anyone who has always struggled with meditation; the wearable gives feedback the cushion never did.

Clinical neurofeedback. qEEG-guided protocols available at neurofeedback clinics. Strongest evidence for ADHD and post-concussion; emerging for general cognitive optimization. The right move if you've plateaued or you're recovering from a concussion or significant head trauma.

HRV breathing tracker — 30 days

Day	5-min session done?	HRV reading (if tracking)
1	☐	
2	☐	
3	☐	
4	☐	
5	☐	
6	☐	
7	☐	
8	☐	
9	☐	
10	☐	
11	☐	
12	☐	
13	☐	
14	☐	
15	☐	
16	☐	
17	☐	

Day	5-min session done?	HRV reading (if tracking)
18	☐	
19	☐	
20	☐	
21	☐	
22	☐	
23	☐	
24	☐	
25	☐	
26	☐	
27	☐	
28	☐	
29	☐	
30	☐	

8.4 — Sauna, Cold, Grounding

The hormetic tier. Stress the system briefly and deliberately, recover, adapt.

Sauna. 15–20 minutes at 170–190°F, 3–4 times per week. Heat shock proteins, cardiovascular adaptation, mental clarity. A home infrared sauna or any commercial

sauna.

Cold plunge or shower. 2–5 minutes, post-workout or AM. Norepinephrine spike, mood lift, brown fat activation. A home cold-plunge unit, or just turn the shower to cold for the last two minutes.

Grounding. Barefoot earth contact, 10–20 minutes per day. Walk in the grass. Stand barefoot on dirt. The cheapest intervention in this entire logbook. A grounding mat if outdoor contact isn't accessible.

> *One of these per day. Rotate. Don't try to do all three every day — you'll quit by week two.*

8.5 — The Environment Audit

Quick yes / no checklist. Aim for all six.

□ Morning sunlight 8–15 minutes within 30–60 minutes of waking

□ Red-spectrum / dim lighting after sundown (or red-spectrum glasses if exposed to screens)

□ Air filtration in bedroom (HEPA + VOC)

□ Reverse-osmosis water with remineralization

□ EMF hygiene in bedroom (router off at night, phone in airplane mode)

□ Red-spectrum glasses if you're at screens after 8 PM

> *Most people optimize their gym before they optimize their bedroom. The bedroom delivers more cognitive return.*

8.6 — The Annual Lab Panel

The labs every Protégé should run every 6–12 months. Bring these to your Rx consult.

Hormones - Total testosterone - Free testosterone - SHBG - Estradiol (sensitive assay) - LH, FSH - DHEA-S - Progesterone (women)

Thyroid - TSH - Free T4, free T3 - Reverse T3 - Thyroid antibodies (TPO, TgAb) if family history

Metabolic - Fasting insulin - Fasting glucose - HbA1c - Comprehensive metabolic panel

Cardiovascular - ApoB - Lp(a) - Full lipid panel - hs-CRP - Homocysteine

Other foundational - Vitamin D 25-OH - Ferritin (men over 50 and post-menopausal women — iron load matters) - CBC with differential

Optional / advanced - NMR LipoProfile - Advanced cardiovascular panel - Coronary CT angiography (if vascular risk) - Multi-cancer blood screen

---

> Two Paths to Act. *Labs feed the decisions. The OTC foundation works without them. The Rx tier does not.*

Part 9 — Resources & Next Steps

You made it to the back of the logbook. Before you close it, do three things: re-score the assessment with 30 days of lived data, decide which Rx paths (if any) you're opening, and write the 90-day plan that takes you out of Month 1 and into the rest of your life.

This part is the reference shelf. Come back to it.

9.1 — The MindSpan Assessment Reference

You took this assessment 30 days ago — on the website, on the app, or on paper. Take it again now. Same 20 prompts, same scoring. The delta is the point.

For each question: circle the number that matches you *today*. Not last month. Not your average. Today. Score guides are anchors — a 1, 2, or 4 is fine when you sit between them.

Mind

Q1 — Cognitive / brain fog *My mind isn't as sharp as it used to be — I forget words mid-sentence, lose focus, or feel mentally foggy more days than not.*

Score guide: 0 = Sharp, focused, fast recall every day 3 = Occasional brain fog or forgetfulness 5 = Daily noticeable cognitive issues that affect work or relationships

This category notes: Mind is the destination of the 4M system — cognitive performance is the end-state we are optimizing for.

Your score: 0 — 1 — 2 — 3 — 4 — 5

Q2 — Sleep *I don't sleep well — trouble falling asleep, waking during the night, or dragging through mornings even after a full night in bed.*

Score guide: 0 = Sleep is solid — I fall asleep fast, stay asleep, and wake rested 3 = Some nights great, some rough 5 = Most nights are bad, I wake up tired, and timing is all over the place

This category notes: The brain cleans itself only during deep sleep. Poor sleep silently degrades hormones, weight, mood, and memory.

Your score: 0 — 1 — 2 — 3 — 4 — 5

Q3 — Hearing, vision & dental *My hearing, eyesight, or dental health has slipped and I haven't had it properly checked or corrected — I strain to follow conversations or read, or I'm ignoring bleeding gums, loose teeth, or overdue dental work.*

Score guide: 0 = All three checked and fully maintained — no strain, healthy gums 3 = Noticeable slippage in one, not yet properly checked or corrected 5 = I regularly miss parts of conversations, avoid reading, or have gum/dental problems I keep putting off

This category notes: Uncorrected hearing loss is one of the largest modifiable dementia risk factors — and gum disease feeds the same inflammation that reaches the brain. The oral microbiome is the front door of the gut-brain axis.

Your score: 0 — 1 — 2 — 3 — 4 — 5

Q4 — Mood *I'm dealing with anxiety, depression, ongoing low mood, or a shorter fuse than I used to have.*

Score guide: 0 = Mood is steady and generally good 3 = Regular low days, irritability, or worry that I notice 5 = Persistent anxiety, low mood, or anger that affects work or relationships most days

This category notes: Depression and anxiety are among the most under-recognized contributors to cognitive decline — and among the most treatable.

Your score: 0 — 1 — 2 — 3 — 4 — 5

Q5 — Social connection *I've become more isolated — fewer real conversations and less regular connection with people I care about.*

Score guide: 0 = Regular, real connection with people I care about 3 = Fewer real conversations than I used to have 5 = Genuinely isolated — days can pass without a meaningful conversation

This category notes: Social isolation carries a dementia risk on par with the big physical factors. Connection is a protocol, not a luxury.

Your score: 0 — 1 — 2 — 3 — 4 — 5

Q6 — Mental challenge *I rarely challenge my mind or learn anything new — most days run on autopilot.*

Score guide: 0 = I learn or build something genuinely new most weeks 3 = Occasional challenge, mostly routine 5 = Full autopilot — nothing new in months

This category notes: The brain follows a ruthless use-it-or-lose-it principle. Novelty and difficulty are what keep the architecture live.

Your score: 0 — 1 — 2 — 3 — 4 — 5

Muscle

Q7 — Movement & strength *I don't move most days, and my strength, muscle, and recovery have been slipping.*

Score guide: 0 = I move daily and train strength regularly — no decline 3 = Inconsistent — some active weeks, some sedentary 5 = Mostly sedentary and clearly weaker than a few years ago

This category notes: Muscle is the engine — and the single best-evidenced lever for both metabolic and cognitive protection.

Your score: 0 — 1 — 2 — 3 — 4 — 5

Q8 — Weight *I'm carrying more body fat than I should — clothes don't fit the way they used to and the scale keeps creeping up.*

Score guide: 0 = At my ideal weight, body composition feels right 3 = About 10-20 lbs heavier than I should be 5 = More than 30 lbs over target with visible belly fat

This category notes: Answer by how your clothes fit and how far you are from your ideal weight — no exact numbers needed.

Your score: 0 — 1 — 2 — 3 — 4 — 5

Q9 — Nutrition *I rely on fast food, processed snacks, or convenience meals more often than I cook real whole-food meals at home.*

Score guide: 0 = Almost everything I eat is whole foods, mostly home-cooked 3 = Mixed — some good days, some takeout / processed days 5 = Fast food, packaged snacks, or convenience meals are my default

This category notes: Food quality is the daily input that drives microbiome diversity, blood sugar, and energy.

Your score: 0 — 1 — 2 — 3 — 4 — 5

Q10 — Pain / injury *Chronic or acute pain, or an injury, keeps me from being as active as I'd like.*

Score guide: 0 = Pain-free and fully active 3 = A nagging issue I work around — it limits some activities 5 = Pain or an injury shuts down most of the activity I'd like to do

This category notes: Pain that blocks movement quietly costs you the muscle, sleep, and mood that protect the brain. It is fixable more often than people assume.

Your score: 0 — 1 — 2 — 3 — 4 — 5

Mitigate

Q11 — Blood pressure *My blood pressure runs high — or I honestly don't know my numbers.*

Score guide: 0 = Measured recently, consistently in range 3 = Borderline readings, or I haven't checked in over a year 5 = Known high blood pressure that isn't controlled — or I've never measured it

This category notes: Midlife blood pressure is one of the strongest predictors of late-life cognitive decline. Not knowing your numbers scores like a problem — because it is one.

Your score: 0 — 1 — 2 — 3 — 4 — 5

Q12 — Blood sugar *I have diabetes or prediabetes — or I've never had my blood sugar checked.*

Score guide: 0 = Checked recently, fully in range 3 = Prediabetic range, or no check in years 5 = Diagnosed diabetes that isn't well controlled — or completely unknown

This category notes: Alzheimer's is often called type 3 diabetes for a reason — the brain runs on glucose signaling, and insulin resistance degrades it.

Your score: 0 — 1 — 2 — 3 — 4 — 5

Q13 — LDL cholesterol *My cholesterol (LDL) is high — or I don't know where it stands.*

Score guide: 0 = Measured recently, in range 3 = Borderline, or no lipid panel in over two years 5 = Known high LDL that isn't being managed — or never tested

This category notes: The arteries that feed the brain are the smallest in the body — they clog first. LDL is a brain number, not just a heart number.

Your score: 0 — 1 — 2 — 3 — 4 — 5

Q14 — Smoking / nicotine *I use tobacco or nicotine regularly.*

Score guide: 0 = Never, or quit years ago 3 = Occasional use, or recently quit 5 = Daily tobacco or nicotine use

This category notes: Smoking is the single most reversible vascular insult on this list — the brain benefits start within weeks of stopping.

Your score: 0 — 1 — 2 — 3 — 4 — 5

Q15 — Alcohol *I drink more than I should — alcohol is a regular part of my week.*

Score guide: 0 = I don't drink 1 = Up to 2 drinks per week 2 = 3-7 drinks per week, never more than 2 per sitting 3 = More than 7 drinks per week, OR more than 2 per sitting regularly 4 = Daily drinker (1-2 drinks per day) 5 = Daily heavy drinker (3+ drinks per day)

This category notes: Alcohol disrupts gut, sleep, hormones, and cognition simultaneously. Cutting back affects every other pillar.

Your score: 0 — 1 — 2 — 3 — 4 — 5

Q16 — Gut health *I deal with bloating, gas, irregular stools, food reactions, or brain fog after meals — my gut clearly isn't happy.*

Score guide: 0 = No gut issues at all 3 = Occasional bloating or food reactions a few times a month 5 = Daily bloating, irregular stools, multiple food sensitivities, foggy after most meals

This category notes: The gut-brain axis sits underneath nearly everything else — fix this first and other categories often improve on their own.

Your score: 0 — 1 — 2 — 3 — 4 — 5

Q17 — Hormones *My hormones feel off — persistent fatigue, low drive, mood swings, poor recovery, or loss of muscle despite trying; for women, also cycle changes, hot flashes, or night sweats.*

Score guide: 0 = Energy, libido, mood, and recovery all feel optimal 3 = Occasional symptoms, especially under stress 5 = Multiple hormone-related symptoms most days affecting daily function

This category notes: Testosterone, estrogen, progesterone, and cortisol balance underlie resilience — proper labs and a physician-led plan beat guessing.

Your score: 0 — 1 — 2 — 3 — 4 — 5

Q18 — Sexual function *My sex drive has dropped and sexual function isn't what it used to be — desire, arousal, or (for men) erection frequency and hardness have declined in the past year or two.*

Score guide: 0 = Drive and function feel completely normal 3 = Some noticeable off-weeks but not a steady pattern 5 = Clear, sustained decline in desire, arousal, or erectile function

This category notes: Declining sexual function is an early-warning signal — the canary in the mine — for hormones, cardiovascular health, and cognitive trajectory. For men it shows up as ED; for women, often as the first sign of the perimenopausal transition. Not a verdict.

Your score: 0 — 1 — 2 — 3 — 4 — 5

Q19 — Environment *I don't get morning sunlight or meaningful outdoor time — I'm under artificial light most of the day, drinking tap water, and surrounded by screens and EMF.*

Score guide: 0 = I get morning sun daily, filtered air and water, low artificial light 3 = Some exposure issues, partially managing 5 = No morning sun, indoors all day under fluorescents/screens, tap water, no air filtration, constant EMF

This category notes: Light, air, water, and EMF compound over years. Lack of morning sunlight is the most overlooked driver of poor sleep and mood.

Your score: 0 — 1 — 2 — 3 — 4 — 5

Motivate

Q20 — Purpose & accountability *I don't have clear health goals or anyone holding me accountable — I know what to do but don't follow through.*

Score guide: 0 = Clear goals, and someone specific holds me to them 3 = Goals exist but nobody checks — follow-through is patchy 5 = No real goals, no accountability, and I know it

This category notes: Knowledge is not the bottleneck — accountability is. Everyone in the program names a specific face they are doing this for.

Your score: 0 — 1 — 2 — 3 — 4 — 5

Already diagnosed? (Yes / No — not scored) *Have you already been diagnosed with mild cognitive impairment, early dementia, Alzheimer's, or Parkinson's?*

This category notes: If you're already in this category, regenerative medicine becomes time-sensitive. A **Yes** routes you to the regenerative-medicine consult path regardless of your other scores.

□ Yes □ No

Scoring rules

1. **Total.** Add all 20 scores. That is your MindSpan Score out of 100 (lower is better).
2. **Band.**

- **0–15 — Sharp.** Protect it.
- **16–35 — Early drift.** Worth getting ahead of.
- **36–60 — Time for a plan.**

- **61–100 — Act now.** While the choice is still yours.

3. **Bonus.** For ranking only, add **+3** to Q16 (Gut health) and **+2** to Q8 (Weight). These carry leverage across every other pillar. A raw 0 stays a 0 — the bonus never lifts an unmarked category.
4. **Top 3.** Rank the twenty scores with bonuses applied. The top three are your priorities for the next 30 days.

Your top-3 priorities

Priority #1 — category: _____
Score (with bonus if applicable): _
What I'm doing about it in the next 30 days:

Priority #2 — category: _____
Score: _
What I'm doing about it in the next 30 days:

Priority #3 — category: _____
Score: _
What I'm doing about it in the next 30 days:

9.2 — The Rx Consult Path

The OTC stack and the lifestyle protocols carry most people a long way. Some need an Rx tier layered on top. Here's the map.

If your audit flagged…	Engage…	Expected workflow
Hormones (low-T symptoms in men, perimenopause/menopause symptoms in women, Q17 or Q18 ≥ 3)	Hormone consult: start with a free care coordinator call at my4mlife.com/consult — labs + coach-led titration. TRT track for men; menopause / hormone-therapy track for women, including non-hormonal options	Full hormone panel first — clinician reviews — Rx if labs, symptoms, and history align
Weight (Q8 ≥ 3, metabolic markers)	GLP-1 telemed consult — semaglutide / tirzepatide / retatrutide candidacy	Intake — labs reviewed — telemed visit — compounded pharmacy ships monthly
Gut (Q16 ≥ 3, chronic GI, food sensitivities)	Prescription gut-barrier consult — proprietary compounded Rx, physician-written	Symptom intake — telemed visit — compounded oral Rx ships in 7-10 days
Sleep (Q2 ≥ 3, clotting/circulation factors)	Sleep Rx consult — nattokinase circulation tier	Sleep intake — coag and lipid review — Rx if indicated
Cognitive optimization (Q1 ≥ 3, no diagnosis)	Nootropic peptide consult — Cerebrolysin / Semax / Selank	Cognitive intake — telemed visit — peptide protocol mailed
Already-diagnosed cognitive decline (a Yes on the final	**Regenerative therapies consult (TIME-SENSITIVE)**	Regenerative protocol intake —

If your audit flagged…	Engage…	Expected workflow
question)		records reviewed — nationwide mobile delivery scheduled
Acute or chronic pain	BPC-157 + TB-500 peptide consult	Pain intake — telemed visit — injectable peptide protocol mailed

Every Rx path begins with a free call with the care coordinator at my4mlife.com/consult. Pick your lane on the form, and bring any labs from the last 90 days.

9.3 — The Weekly Zoom Schedule

The cohort runs on a weekly rhythm. Three touchpoints. None of them optional if you want the work to stick.

- **Weekly Zoom — scheduled based on demand** — Live with Dr. TJ. Cohort Q&A, protocol updates, case discussions. When a recording is posted, it appears on your app home screen. If your schedule doesn't allow you to attend, email your question to support@my4mlife.com and watch the replay.
- **Sunday Reflection — solo** — On paper, in the app, or in the logbook. Twenty minutes. The reflection prompts are at the end of each week.
- **Weekly Adherence Scorecard** — The tear-out page at the end of each week. Tape it to your bathroom mirror or screenshot it for your phone lock-screen. Score yourself at end of week.

12-Week Zoom Log

Wk	Date	What I brought	What I took away	One adjustment for next week
1				
2				
3				
4				
5				
6				
7				
8				
9				
10				
11				
12				

The people who fill this in are the ones who finish the program.

9.4 — Where to Find Everything

Resource	Where
The My4MLife app	https://app.my4mlife.com — sign in with your email, OTP code
The free 4M Assessment	https://my4mlife.com/assessment
The book — *Begin with the End in Mind*	Protégé welcome gift by email, or standalone purchase at https://my4mlife.com/book
Regenerative therapies	See my4mlife.com for the current regenerative protocol and nationwide mobile delivery details
The blog — long-form articles	https://my4mlife.com/blog
Weekly Zoom links	Sent by email before each call
The current products	https://my4mlife.com/stack
Rx Consult booking	https://my4mlife.com/consult

Support

- **General questions** — support@my4mlife.com
- **Billing / orders** — billing@my4mlife.com
- **Rx / clinical** — through your care coordinator at my4mlife.com/consult (HIPAA)
- **Cohort + Zoom** — sent by email each week

9.5 — The 90-Day Plan Template

Month 1 was the lived foundation. Months 2 and 3 tighten what's working. Month 4 and beyond is where the Advanced Layer from Part 8 gets evaluated.

Write this plan now, while the 30 days are fresh.

Months 2-3: Continue + Tighten

What's staying from Month 1 — the behaviors I will not negotiate:

>

What's getting tighter — the specific behaviors and metrics I'm sharpening:

Behavior / metric	Month 1 baseline	Month 3 target

The one behavior I keep falling off — and the 30-day recommit move I'll use when I do:

> *Falling off: _____*
>
> *Recommit move (pick ONE behavior, run it 3 days, then add the next):*
>
> ---

My accountability target for the next 90 days — the specific face I'm doing this for:

> ---

Month 4 onwards: Add (or not)

The Advanced Layer in Part 8 is optional. Don't add it until the foundation is locked. Use this page to write down what you're considering and what conditions you want met before you add it.

Item I'm considering: _____

> *Conditions I want met first (90-day adherence ≥ 80%, labs in range, sleep score, etc.):*
>
> ---
>
> ---
>
> ***Earliest review date:*** ___

Item I'm considering: _____

> *Conditions I want met first:*
>
> ***Earliest review date:*** ___

Item I'm considering: _____

> *Conditions I want met first:*
>
> ***Earliest review date:*** ___

What I am explicitly NOT adding in the next 90 days — and why:

>

9.6 — A Closing Letter from Dr. TJ

You made it through Month 1.

I don't say that as a participation trophy. I say it because most people who buy a program like this never get past the first two weeks. You did. You ran the protein-first rule. You closed the eating window. You walked into morning sun. You loaded the stack. You filled in the trackers. You took your scorecard seriously enough to keep coming back to it.

The people who finish Month 1 are not the people who started it. The person who started had a vague sense that things were sliding. The person finishing has data, a stack, a face they're doing this for, and 30 days of evidence that they can run a protocol and stick to it. That's not nothing. That's the foundation everything else is built on.

You are not doing this alone. The cohort runs alongside you. I'm on the weekly Zoom. Sunday nights you reflect. The other people in your cohort are in the same fight on the same week. When you fall off — and you will, that's not failure, that's the work — the recommit procedure is right there. One behavior. Three days. Then the next.

Begin with the end in mind.

Don't lose your identity and your dignity while you still have a choice.

Mind is the destination.

I'll see you on the next Zoom. Bring a question.

— Dr. TJ

About the Author

Dr. TJ Mundheim is the founder of My4MLife. He built the program because he is living it — every protocol in this logbook is one he runs himself. He is here to teach it, refine it, and run it alongside the people who choose to plug in.

Connect

- **Free 5-minute 4M Assessment:** my4mlife.com/assessment
- **The My4MLife app:** app.my4mlife.com
- **The book — *Begin with the End in Mind*:** included free with every Protégé signup
- **Regenerative therapies:** nationwide mobile delivery for the already-diagnosed — see my4mlife.com for the current protocol
- **Support:** support@my4mlife.com

Begin with the end in mind.

Don't lose your identity and your dignity while you still have a choice.

— Dr. TJ Mundheim My4MLife · 2026